

Endorsement

DRN :STB-06-2026-000117-E

EXECUTIVE SUMMARY FOR THE UNDERSECRETARY

SUBJECT	:	SUBMISSION OF PILOT IMPLEMENTATION MANUAL OF THE PROJECT ARUGA
DATE	:	01 JUNE 2026

Summary:

This is to respectfully submit to the Undersecretary the Pilot Implementation Manual of the Project Aruga. This manual will serve as a guide to the implementers of the Project in all pilot implementation sites which includes the procedural processes of implementing the project activities.

Action Needed:

For the Undersecretary's approval and further instruction.

LIST OF ATTACHMENTS

- 1) Memorandum Transmittal; and
- 2) Pilot Implementation Manual of The Project Aruga

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FOR USE OF THE OFFICE OF THE UNDERSECRETARY

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DRN: STB-06-2026-000117-E**MEMORANDUM**

FOR : UNDERSECRETARY EDUARDO M. PUNAY
Innovations and Program Development Group

FROM : THE DIRECTOR III

**SUBJECT : SUBMISSION OF PILOT IMPLEMENTATION MANUAL OF THE
PROJECT ARUGA**

DATE : 01 JUNE 2026

This is to respectfully submit to the Undersecretary the Pilot Implementation Manual of Project ARUGA.

The Manual serves as the primary reference guide for the implementation of Project ARUGA across nine (9) pilot regions. It provides the standards and procedures for the implementation of project activities, sets the parameters for budget allocations under the various project mechanisms, and clarifies the roles and responsibilities of the Department, National Government Agency and Civil Society Organization partners, and Local Government Units (LGUs) serving as implementation sites.

The Manual is intended to promote a uniform and coordinated approach in project implementation and ensure the effective delivery of interventions and services to target beneficiaries.

For the Undersecretary's information and further instructions.

Thank you.

MARILYN B. MORAL

APPROVED/ DISAPPROVED BY:

EDUARDO M. PUNAY
Undersecretary for Innovations and Program Development

Manual of Enhanced Guidelines



PROJECT
Aruga
by DSWD

PROJECT ARUGA

PILOT IMPLEMENTATION MANUAL

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TABLE OF CONTENTS

LIST OF ABBREVIATIONS	1
CHAPTER I	2
RATIONALE	2
CHAPTER II	4
LEGAL BASES	4
DEFINITION OF TERMS	5
CHAPTER III	8
PROJECT DESCRIPTION	8
OBJECTIVES	8
SCOPE AND COVERAGE	8
PROJECT DESCRIPTION AND OBJECTIVES	8
CHAPTER IV	12
PROJECT APPROACHES	12
CHAPTER V	13
IMPLEMENTING PROCEDURES	13
CHAPTER VI	37
REPORTING AND DOCUMENTATION	37
CHAPTER VII	38
INSTITUTIONAL ARRANGEMENTS	38

LIST OF ABBREVIATIONS

AICS	Assistance to Individual in Crisis Situations
BCPC	Barangay Council for the Protection of Children
COP	Compliance Officer for Privacy
CO	Central Office
CIP	Crisis Intervention Program
CWD	Children with Disability
CHO	City Health Office
DARECARE	Development Assessment Rehabilitation and Empowerment of Children with Disabilities
DPO	Data Protection Officer
DPA	Data Privacy Act
DEPED	Department of Education
DOH	Department of Health
DILG	Department of Local Government
DSA	Disability Support Allowance
DSWD	Department of Social Welfare and Development
DPO	Data Protection Officer
EDPID	Early Detection Prevention and Intervention for Disability
EO	Executive Order
FO	Field Office
FO PMT	Field Office Project Management Team
GL	Guarantee Letter
ILRC	Inclusive Learning Resource Center
IS	Information System
LCE	Local Chief Executive
LGU	Local Government Unit
LPI	Local Project Implementers
LSWDO	Local Social Welfare and Development Officers
MOA	Memorandum of Agreement
MOV	Means of Verification
NCDA	National Council on Disability Affairs
NDA	Non-Disclosure Agreement
SBCC	Social Behavior Change Communication
SLP	Sustainable Livelihood Program
ST	Social Technology
STB	Social Technology Bureau
STU/S	Social Technology Unit/ Section
TESDA	Technical Education and Skills Development Authority
UN	United Nations
UNICEF	United Nations Children's Fund
PID	Pilot Implementation Division
PSA	Philippine Statistics Authority
PDAO	Persons with Disabilities Affairs Office
PDPB	Policy Development and Planning Bureau
PSB	Protective Services Bureau
PMT	Project Management Team
PHILHEALTH	Philippine Health Insurance Corporation
PHILSYS	Philippine Identification System
PWD	Persons with Disability
PRPWD	Philippine Registry of Persons with Disability

CHAPTER I

RATIONALE

The Philippines has been one of the first countries to recognize the rights of Persons with Disabilities with a law on accessibility enacted in 1983, followed by the Magna Carta for Persons with Disabilities in 1992 (Republic Act No. 7277), which was amended accordingly to facilitate compliance with the Act and to achieve its objectives. The Philippines also ratified the United Nations (UN) Convention on the Rights of Persons with Disabilities in 2008 and the UN Convention on the Rights of the Child in 1990, alongside regional commitments, including the Incheon Strategy “to Make the Right Real” for Persons with Disabilities in Asia and the Pacific.

Despite a strong legal basis, implementation of the rights of Persons with Disabilities is falling behind, and supporting programs lack adequate coverage and depth. The emerging picture is that there is still lack of proper accountability and coordination in the provision of services for Children with Disabilities, that some of the services have low coverage (e.g., lack of assistive devices), and that the discount and preferential treatment that persons with disability should have (not paying VAT and 20% discount on medicines, medical fees, transportation, restaurant and certain recreational expenses, together with a 5% discount on essential items) often does not materialize.

Based on the data from the Department of Social Welfare and Development (DSWD), there are 89,261 Children with Disabilities belonging to poor households in the Philippines. Further, the current database of the DSWD only includes indicators for physical disability; thus, this number may still increase if psychosocial, cognitive, and other forms of disabilities are included.

The DSWD and the United Nations Children's Fund (UNICEF) conducted the Study on the Cost of Raising Children with Disabilities in the Philippines, which was completed and launched in December 2022. The study aimed to quantify the extra monetary costs faced by households with Children with Disabilities and define such extra costs in relation to achieving living standards in comparison to other households with children. Although the study is not a disability prevalence survey, selecting children with a disability identification (ID) card using a rigorous sampling strategy provided an estimate of the total number of children with a disability ID card in the Philippines, which is estimated to be 325,000 children. Moreover, it also revealed that the poverty rate is 50% higher for households with Children with Disabilities.

Accordingly, the main source of extra costs of raising Children with Disabilities is health-related expenses. Estimates point to an extra cost of raising Children with Disabilities, requiring equivalence scales for children with functional difficulties that range from 1.4 to 1.8, implying that such children require expenditures that are 40% to 80% higher than those for other children. Against an average extra cost at the household level of 10% to 20%, even for those who have and use the disability card or receive cash transfers, the support is only a fraction of what would be required. Moreover, the study also found evidence of substantial unmet needs, which do not materialize in costs and would require service provision. These unmet needs include the failure to enroll in school, evidence of forgotten health treatment and consultations, and a lack of provision of assistive devices.

The Department of Social Welfare and Development (DSWD), in partnership with the UNICEF Philippine Country Office, developed and pilot-tested Project Aruga in four (4) local government units, namely: Laoag City, Ilocos Norte; Puerto Princesa, Palawan; San Jose de

Buenavista, Antique; and Sta. Cruz, Davao del Sur. In 2026, with the support of the Australian Government, the Project was evaluated, and it was concluded that Project Aruga has strong potential for expansion. However, scaling up will require a clearer delivery system, simpler documentation and cash support processes, stronger referral tracking and partnerships, better support for frontliners and primary caregivers, and stronger integration into LGU policy, budget, and coordination systems.

It is in this context that the Social Technology Bureau (STB), as the program development arm of the DSWD, has redesigned the Project's components and its corresponding mechanisms by iterating the processes of Project design and testing as further provided by DSWD Administrative Order No. 14, series of 2018 or the "Omnibus Guidelines in Social Technology Development". The Project is deemed for implementation in five (5) more Local Government Units with the highest concentration of Children with Disabilities in order to validate the reliability and validity of the mechanisms as recommended by the Project Evaluation prior to its full institutionalization.

CHAPTER II

LEGAL BASES

A. International Instruments

- 1) **Universal Declaration of Human Rights (1948)** highlights the rights of all individuals, particularly the Persons with Disabilities;
- 2) **United Nations Convention on the Rights of Persons with Disabilities (CRPD)** provides the core international treaty, adopted in 2006, ensuring disabled individuals or Persons with Disabilities to enjoy all human rights on an equal basis with others, including legal capacity, accessibility, and reasonable accommodation;
- 3) **United Nations Convention on the Rights of the Child** defines the core rights of the child namely, survival, protection, development, and participation; and
- 4) **The Incheon Strategy “to Make the Right Real” for Persons with Disabilities in Asia and the Pacific** sets regionally agreed, disability-inclusive development goals for 2013–2022 and aims to improve the lives of 650 million+ people with disabilities through 10 goals, 27 targets, and 62 indicators, covering areas like poverty reduction, employment, and political participation.

B. National Laws and Issuances

- 1) **Presidential Decree 603** otherwise known as the “**Child and Youth Welfare Code**” highlights the rights of children and youth, recognizing their inherent worth and dignity, and provides that all children with disabilities are entitled to treatment, education, and care suited to their needs;
- 2) **Republic Act No. 7277** or the “**Magna Carta for Persons with Disabilities**” recognizes Persons with Disabilities’ right to equal opportunities and prohibits acts of discrimination in employment, education, health, transportation, and public services;
- 3) **Republic Act No. 7160** or the “**Local Government Code of 1991**” transfers significant powers, responsibilities (health, social welfare, public works, etc.) and resources from the national government to local government units for basic service delivery and development;
- 4) **Republic Act No. 10070, amending Republic Act No. 7277** intensifies the systematic and localized mechanisms in providing services among Persons with Disabilities at the local government level through the creation of Persons with Disabilities Affairs Office in every province, city, and municipality in the country;
- 5) **Republic Act No. 11223** or the “**Universal Health Care Act**” provides that all Filipinos should have access to a comprehensive set of quality, cost-effective, promotive, preventive, curative, rehabilitation, and palliative health care services without causing financial problems and hardships and

prioritizes the needs of the population who cannot afford services;

- 6) **Republic Act No. 10754** or the “**Act Expanding the Benefits and Privileges of Persons with Disabilities**” ensures persons with disability the opportunity to participate fully into the mainstream of society by granting them at least twenty (20%) discount and exemption from the value-added tax on the sale of certain goods and services identified under Republic Act No. 9442 for the exclusive use, enjoyment or availment of persons with disability;
- 7) **Republic Act No. 10533** or the “**Enhanced Basic Education Act of 2013**” creates a functional basic education system that will develop productive and responsible citizens equipped with the essential competencies, skills, and values for both lifelong learning and employment and that will be inclusive for learners with disabilities;
- 8) **Republic Act No. 11650** or the “**Instituting a Policy of Inclusion and Services for Learners with Disabilities in Support of Inclusive Education Act**” mandates that all public and private schools provide inclusive education to learners with disabilities (LWDs) by ensuring free, quality, and equitable access to basic education, establishing Inclusive Learning Resource Centers (ILRCs) nationwide to provide support, early intervention, and individualized education plans;
- 9) **Republic Act No. 6727** or the “**Wage Rationalization Act of 1989**” established a decentralized, regional-based mechanism for setting minimum wages based on regional living costs, poverty thresholds, and economic conditions, rather than a single national rate;
- 10) **Republic Act No. 10173** otherwise known as “**Data Privacy Act of 2012**” ensures that the personal data and information acquired from individuals must be secured and protected; and
- 11) **Republic Act No. 11315** or the “**Community-based Monitoring System Act**” mandates the nationwide collection of disaggregated, localized, and up-to-date data to improve poverty alleviation efforts and development planning in the Philippines.

C. Department Issuance

- 1) **DSWD Administrative Order No. 02, series of 2024** or the “**Adoption and Implementation of DSWD Strategic Plan for 2024-2028**” ensures the availability and accessibility of social welfare programs and services that clients' immediate needs towards strengthening social protection systems.

DEFINITION OF TERMS

- 1) **Auxiliary Caregivers** – for the purpose of this Pilot Implementation Manual, refers to identified willing individuals within the community who will be trained to assist the primary caregivers in providing care and support for children with disabilities.
- 2) **Auxiliary Services** – for the purpose of this Pilot Implementation Manual, refers to the additional support services that complement the primary activities of Project

Aruga enhancing its overall impact and ensuring that beneficiaries receive holistic care and support.

- 3) **Case Management** – refers to a systematic process used to assist beneficiaries in overcoming challenges and improving their well-being that aims to empower beneficiaries by helping them navigate and access a range of social services, providing tailored support, and ensuring that interventions are responsive to their unique needs.
- 4) **Case Manager** – refers to the Social Worker identified by the local government unit to facilitate and oversee the case management process of children with disabilities and their families following DSWD's established standards, policies, and procedures provided by DSWD Pilot Implementation Manual No. 02, series of 2008.
- 5) **Disability Support Allowance** – for the purpose of this Pilot Implementation Manual, refers to the monetary-based assistance amounting to P2,000.00 which will be provided to eligible children with disabilities and their families regularly to augment the extra costs to meet their basic needs, along with health, education, nutrition, transportation and among others.
- 6) **Home Care and Community Support Services** – for the purpose of this Pilot Implementation Manual, refers to the provision of support by the Auxiliary Caregivers to the families of children with disabilities in providing care to the child, such as ensuring the child eats, good hygiene, among others, that would enable their parents and/or recognized caregivers to attend to work.
- 7) **Local Project Implementers** – for the purpose of this Pilot Implementation Manual, refers to the focal persons in the local government unit and partner stakeholders at the provincial and regional level who are responsible for executing the project and are often the key actors who interact directly with the target communities, delivering services, support, and interventions based on the project's goals.
- 8) **Philippine Registry of Persons with Disability (PRPWD)** – the national database managed by the Department of Health (DOH) designed to collect, store, and manage information on persons with disabilities across the country.
- 9) **Persons with Disability** – refers to individuals with long-term physical, mental, intellectual, or sensory impairments that, when interacting with various barriers, hinder their full, equal participation in society.
- 10) **Project Aruga Profiling and Assessment Tool** - a baseline and assessment tool that aims to collect identifying information and current conditions of potential children with disabilities and their household in consideration to access to Housing, Sanitation and Water, Health, Education and Economic Capacity.
- 11) **Project Management Team** – for the purpose of this Pilot Implementation Manual, refers to the team within the DSWD who are responsible for overseeing the successful planning, implementation, monitoring, and evaluation of Project Aruga, ensuring that all objectives are met, and the children with disabilities and their families receive the intended services and support.
- 12) **Reasonable Accommodation** – refers to necessary and appropriate modifications or adjustments made in the workplace, educational institutions, or other public or private services to ensure that individuals with disabilities or specific needs have

equal access to opportunities without imposing disproportionate or undue burden on the entity providing them.

- 13) **Referral Network** – a structured system of service providers and organizations that collaborate to ensure that children and their families can access the appropriate services, support, and resources they need.
- 14) **Referral Pathway** – refers to a coordinated system of processes that guides children with disabilities and their families to access appropriate services and support. The pathway outlines the steps and connections needed to ensure that the children with disabilities and their families receive care and support from various partners that are part of the referral network.
- 15) **Social and Behavior Change Communication** – refers to a collection of communication approaches, activities, and tools used to positively influence behaviors among the project beneficiaries, general public, and partner stakeholders.
- 16) **Social Case Management** – refers to the structured and systematic process used by social workers to assess, plan, implement, coordinate, monitor, and evaluate services for the children with disabilities and families in need.

CHAPTER III

PROJECT DESCRIPTION

OBJECTIVES

This Pilot Implementation Manual seeks to:

- 1) Provide standards and procedures for implementation the Project Aruga in nine (9) regions;
- 2) Set parameters on budget allocations in the implementation of the different mechanisms under the Project; and
- 3) Clarify the roles and responsibilities of the DSWD, National Government Agency and Civil Society Organization partners and Local Government Units (LGUs) as implementation sites.

SCOPE AND COVERAGE

This Pilot Implementation Manual is limited in the implementation of Project Aruga in nine (9) LGUs duly endorsed by DSWD Field Offices I (Ilocos Region), II (Cagayan Valley Region), III (Central Luzon), IVA (CALABARZON) IVB (MIMAROPA) V (Bicol Region), Region VI (Western Visayas), Region XI (Davao Region) and National Capital Region.

The Project shall initially be implemented in the identified LGUs, with phased expansion to additional regions in the succeeding years, subject to evaluation results, resource availability, and issuance of appropriate guidelines.

These LGUs must have met the following criteria as pilot implementation site:

- a) High concentration of target beneficiaries as provided by the Philippine Registry of Persons with Disability of the DOH;
- b) Political support for the Project; and
- c) Availability of resources within the LGU.

PROJECT DESCRIPTION AND OBJECTIVES

a) **Description:**

The Project Aruga is a community-based model of intervention that provides disability-inclusive social protection to Children with Disabilities and their families to address substantial unmet needs and provide comprehensive packages of social and health interventions realizing client-centered, family-focused and community-oriented approaches. The Project shall entail the provision of Disability Support Allowance (DSA) to the Parents and/or recognized Caregivers of Children with Disabilities in relation to the disability-specific support needs.

b) **Objectives:**

Children with Disabilities are mainstreamed in the Community with the support of their Parents and/ or Recognized Caregivers

Specifically, the ST aims to:

- 1) Enhance the detection, prevention and intervention through service provider linkages and networks;
- 2) Provide Disability Support Allowance to augment the cost of care of Children diagnosed with disability;
- 3) Increase access to other services such as health, education, skills development, livelihood, employment, and social insurance among Children with Disability and their families through strengthening local mechanisms;
- 4) Capacitate immediate family members and auxiliary caregivers to deliver Home Care and Community Support Services; and
- 5) Advocate for inclusive communities that will support the development of Children with Disabilities.

c) **Components:**

1. **Early Detection, Prevention, and Intervention for Disability** – ensures early detection, prevention, and intervention for disability and facilitates assessment-seeking behaviors and provides appropriate referral to relevant institutions, whether public or private, for more accurate identification of the needs, strengths, and challenges of Children with Disabilities across health, education, and assistive device provision.
2. **Disability Support Allowance** – financial support also known as *Disability Support Allowance* (DSA) shall be provided for the Children with Disabilities through their family representatives to augment the extra costs of families with Children with Disabilities who incur high cost-of-living expenses to meet their basic needs, along with health, education, nutrition, and transportation based on the disability-related extra costs.
3. **Auxiliary Services** – part of the case management intervention of the Local Social Welfare and Development Office, services will be provided to the Children with Disabilities and their families based on their assessed needs. The Project shall strengthen and harmonize the efforts of different government agencies towards the delivery of the following services to the Children with Disabilities and their families:
 - a. **Social Welfare Services** – the Local Government Unit shall extend to the eligible Children with Disabilities and their families interventions that will ensure their protection, rehabilitation, participation and optimum development and these include but are not limited to: a) Assistance to Individuals in Crisis Situations (AICS), b) Counselling sessions, c) Family Strengthening activities, d) Psychosocial support and other relevant services.
 - b. **Health Services** – necessary referrals will be facilitated to ensure that Children with Disabilities can access a range of services offered by the Local Government Unit, government-managed hospitals, clinics, and other relevant institutions and these include: a) preventive care, b) diagnostic care, c) rehabilitation and therapies, d) treatment, and e) provision of medicines related to their impairments or health conditions. This approach aims to provide comprehensive

support, addressing the diverse health and well-being needs of Children with Disabilities.

- c. **Educational Services** – Children with Disabilities may also be referred to the programs implemented by the Department of Education for inclusive education through the implementation of the Republic Act No. 11650, otherwise known as the "Instituting a Policy of Inclusion and Services for Learners with Disability in Support of Inclusive Education Act". Inclusive education will address and respond to the diversity of needs of all learners by moving towards the end-goal of full participation, presence and achievement in learning cultures and communities, which involves accommodation, modification, adaptation, and individualization in content, approaches, structures, and strategies.
 - d. **Social Insurance Services** – Pursuant to the Republic Act No. 11228 or "An Act Providing Mandatory PhilHealth Coverage to Persons with Disabilities", the Project ensures the mandatory coverage for all Children with Disabilities from basic care to catastrophic health package through PhilHealth;
 - e. **Skills Development and Enhancement** – through partnership with Technical Education and Skills Development Authority (TESDA), Project Aruga shall facilitate the provision of available training programs, including community-based training, to caregivers and families of Children with Disabilities to strengthen their capacities for livelihood, self-employment, and productive community participation. These training programs shall be responsive to the needs and available opportunities within the community and may include technical assistance such as training modules, qualified trainers, competency assessment and certification, monitoring and evaluation, and, when feasible, starter tool kits to support the application of acquired skills.
 - f. **Livelihood/ Employment Facilitation Services** – opportunities may be created under the Project to assist the family members, specifically their Parent and/ or Recognized Caregiver, of the Children with Disabilities in working age to be provided with job facilitation and/or self-employment to foster socio-economic participation as part of the disability-inclusive social protection through the Sustainable Livelihood Program of the DSWD or an analogous program within the Local Government Unit.
- 4. **Family Support Services** – Capacitating Parents and/ or Recognized Caregivers of Children with Disabilities through peer-support sessions, family strengthening activities, counseling, and other relevant interventions that promote protection, rehabilitation, and optimum development.
 - 5. **Advocacy Campaigns** – Mobilizing communities and deploying Social Behavior Change Communication materials aimed at reducing stigma, promoting disability inclusion, and building supportive environments for Children with Disabilities.

d) Target Beneficiaries:

1) Primary Clients

The primary clients of Project Aruga are Children with Disabilities from poor and vulnerable households, identified through the Registry of Persons with Disabilities maintained by the Persons with Disabilities Affairs Office at the Local Government Unit level, supplemented by relevant national databases. All beneficiaries are subject to validation and registration under PhilSys in accordance with Republic Act No. 11055 or the “Philippine Identification System Act”.

The Parents and/or Recognized Caregivers of Children with Disabilities who are biological parents or guardians recognized legally or by the community to provide care and support to the child diagnosed with disability are also primary clients under the Project.

2) Secondary Clients

The secondary clients of Project Aruga are Local Government Units and Local Stakeholders in the pilot areas, service providers including social workers and teachers, and the general public. These groups benefit indirectly through strengthened local systems, improved capacity for disability-inclusive service delivery, and broader community awareness of the rights and needs of Children with Disabilities.

CHAPTER IV

PROJECT APPROACHES

- 1) **Client-Centered** – the Project adopts a client-centered approach by ensuring that all interventions are responsive to the individual needs, rights, and aspirations of each child with disability. Intervention shall be aligned with individualized case management plans developed in coordination with the child, their parents and/or recognized caregivers, consistent with the principles of dignity, autonomy, and non-discrimination under the United Nations Convention on the Rights of Persons with Disabilities and the Republic Act No. 11650. This approach also ensures that services are tailored according to assessed needs and the best interests of the child.
- 2) **Family-Focused** – this recognizes the family as the primary unit of care and socialization of the children with disabilities. It shall strengthen the capacity of parents and/or recognized caregivers to provide responsive and home-based and community support through structured case management, counseling, family strengthening sessions, and disability-sensitive parenting education. The provision of DSA shall be directed to families to reduce disability-related economic strain and improve caregiving conditions. This approach also aligns with ecological systems theory, which situates child development within interconnected family and environmental systems by supporting caregiving environment from the child's parents and/or recognized caregivers gearing towards stability, protection, and long-term developmental outcomes.
- 3) **Community-Oriented** – this will enable local stakeholders to strengthen inter-agency collaboration and institutionalize referral systems at the barangay, municipal, regional, and national levels. The Data Management and Referral Network shall harmonize information systems and promote coordinated service delivery among Local Government Units, health institutions, schools, and social protection agencies. Community volunteers shall be mobilized and capacitated as Auxiliary Caregivers through an Asset-based Community Development approach, and stigma reduction initiatives shall likewise be implemented through Social Behavior Change Communication strategies.
- 4) **Strengths-based** – this identifies and builds upon the capacities of children with disabilities, their families and/ or recognized caregivers, and communities rather than focusing solely on deficits or their impairments. Assessment processes shall document protective factors, adaptive skills, social support systems, and community assets that can be leveraged to support participation and inclusion. Interventions, including livelihood facilitation and educational referrals, shall promote empowerment, resilience, and self-determination. This approach ensures that children with disabilities are recognized as rights-holders with potential and agency, thereby aligning social protection with inclusive development principles.
- 5) **Disability-Mainstreaming** – this refers to the systemic integration of disability inclusion across all components of the project to ensure that children with disabilities and their families can access social protection and related services. This is also carried out through the inter-agency collaboration at the local, regional, and national levels, alongside the provision of DSA to help address disability-related costs for basic needs, health care, and education. The project also strengthens local mechanisms that link families to essential services. Further, parents, family members, and auxiliary caregivers are capacitated to provide appropriate home care and community-based support.

CHAPTER V

IMPLEMENTING PROCEDURES

A. PRE-IMPLEMENTATION PHASE

1) Ceremonial Signing of Memorandum of Agreement with Pilot Local Government Units

- 1.1 The Project Focal Person from the STB shall transmit, through official channels, the template of the Memorandum of Agreement (Appendix A) approved by the Legal Service of the DSWD Central Office to the DSWD Field Office;
- 1.2 After its receipt in the DSWD Field Office, the STU/S shall be responsible for routing the MOA to the Legal Section/ Unit and pilot implementation site for further comments and inputs;
- 1.3 The implementation site shall be given a maximum of twenty (20) calendar days from the receipt of the MOA to provide their comments and inputs, in form and substance, that shall be submitted to the DSWD Field Office through official channels for its consideration and further disposition;
- 1.4 The minimal comments, in form and substance, shall be considered by the STU/S with the concurrence of the Legal Unit/ Section of the Field Office. However, if comments and inputs warrant major revisions on the provisions of the MOA, the STU/S shall transmit the comments and inputs to the STB for proper clearance by the Legal Service of the DSWD Central Office;
- 1.5 The STB shall assess the relevance and appropriateness of the comments and inputs of the Field Office and/ or implementation site and shall be responsible for securing the clearance of the Legal Service, if deemed appropriate. Once cleared, the STB shall transmit the same to the Field Office for onward submission to the pilot implementation site;
- 1.6 The MOA may be presented by the STU/S to the *Sangguniang Panlungsod/ Bayan* that would assure the issuance of a local resolution that will Authorize the Local Chief Executive (LCE) to Enter into a MOA with the DSWD Field Office for the pilot implementation of the Project;
- 1.7 The STU/S shall notify the STB, through the official channels, once the date and venue of the Ceremonial Signing of the MOA has been determined between the Field Office and the implementation site;
- 1.8 The LCE, DSWD Field Office Director, and Cluster Head of the Innovations and Program Development Group shall serve as the primary signatories of the MOA. While the Director of the STB

and Head of the Local Social Welfare and Development Office (LSWDO) shall serve as witnesses. They may designate representative/s in the event of unavailability for the Ceremonial MOA Signing;

- 1.9 The Ceremonial MOA Signing shall be led by the STU/S in close coordination with the LSWDO and Persons with Disability Affairs Office (PDAO) following the Program of Activities (Annex A) as standardized by STB;
- 1.10 The STU/S shall procure the needed logistical requirements, subject to the availability of funding, for the conduct of the activity. *Refer to the DSWD CO or FO level Guidelines on the Procurement of Goods and Services under the implementing rules and regulations of Republic Act No. 12009 for further guidance;*
- 1.11 The pilot implementation site and DSWD Field Office may recommend to the STB the routing of the MOA for approval in lieu of a Ceremonial Signing, subject for consideration of the Office of the Undersecretary for Innovations and Program Development. The document routing must be completed by all parties within 20 calendar days; and
- 1.12 The STU/S shall facilitate the notarization of the duly approved MOA within seven (7) working days after its Ceremonial Signing. The STU/S shall furnish the implementation site and the STB with the notarized MOA for filing for future reference. *Refer to the implementing rules and regulations of Republic Act No. 9470 or the "National Archives of the Philippines Act" or other relevant issuances for the filing and archival of the MOA as binding document.*

2) Formation of Local Project Implementers

- 2.1 Upon the approval of the MOA, the STU/S shall coordinate with the LSWDO and PDAO for the organization of the Local Project Implementers;
- 2.3 The Local Project Implementers shall ensure that the activities under the Project shall be implemented within the LGU. Its minimum composition shall include focal persons from the: LSWDO, PDAO, Local Health Office (LHO), Early Childhood Care and Development (ECCD), Public Employment Service Office (PESO), Department of Education (DepEd) - Schools Division Office, and etc. and may be expanded to private entities and Civil Society Organizations that would further the interventions under each activity of the Project;
- 2.3 Each of the offices shall designate one (1) focal person or representative, regardless of employment status within the LGU, who will be part of the Local Project Implementers. The concerned office will endorse the name of the assigned focal,

together with employment position, e-mail address and contact number to the LSWDO or PDAO as its default Secretariat;

- 2.4 A Project Orientation among the members of the proposed Local Project Implementer shall be organized by the LSWDO and PDAO and the STU/S shall serve as Resource Person. The STU/S may procure the needed logistical requirements, subject to the availability of funding, for the conduct of the activity;
- 2.5 The Terms of Reference (TOR) of Local Project Implementers (Appendix B) crafted by the STB shall be thoroughly presented among the proposed members of the Local Project Implementers. The Roles and Responsibilities as indicated in the TOR of each office may be expanded by the members of the Local Project Implementers, as deemed necessary and appropriate.
- 2.6 The LSWDO shall advocate to the LCE or *Sanguniang Panlungsod/ Bayan*, as deemed appropriate, the issuance of an Executive Order or Local Resolution for the constitution of the Local Project Implementers. The LSWDO or PDAO may conduct a separate Project Orientation to the LCE and/ or *Sanguniang Panlungsod/ Bayan* if the circumstances require;
- 2.7 A copy of the local policy constituting the Local Project Implementers shall be forwarded by the PDAO to the STU/S for the reference of the DSWD Field Office; and
- 2.8 The Local Project Implementers shall meet regularly on a monthly basis or as deemed necessary. The STU/S may procure the needed logistical requirements, subject to the availability of funding, for the conduct of the activity.

3) Formation of Field Office Project Management Team

- 3.1 The STU/S shall initiate the formation of the Project Management Team (PMT) at the level of the Field Office. The PMT shall be responsible for coordination, promotion and efficient delivery of services under the Project in the level of the region;
- 3.2 The PMT at the Field Office shall be led by the STU/S including its secretariat functions. The minimum members of the PMT shall include: STU/S, Crisis Intervention Section/ Unit, Sustainable Livelihood Program - Regional Program Management Office (RPMO), KALAHI CIDDs - RPMO, Social Marketing Unit/ Section, Persons with Disabilities Focal Person, Secretariat of the Regional Council for Disability Affairs, and etc;

- 3.3 The proposed members of the PMT must be duly endorsed by their respective Unit/ Section/ Office Heads regardless of employment status. The Head of the Units/ Sections or Office shall ensure that their representation to the PMT shall be sustained in the course of Project implementation;
- 3.4 Upon the initial confirmation of the proposed members, the STU/S shall orient the PMT on their Roles and Responsibilities through its approved TOR (Appendix C) crafted by the STB. The Roles and Responsibilities as indicated in the TOR of each office may be expanded by the members of the PMT, as deemed necessary and appropriate;
- 3.5 The PMT shall be constituted through a Special Order or Regional Special Order, whichever is applicable. The Special Order or Regional Special Order shall be updated by the STU/S on an annual basis or as deemed necessary; and
- 3.6 The PMT shall meet regularly on a monthly basis or as deemed necessary. The STU/S may procure the needed logistical requirements, subject to the availability of funding, for the conduct of the activity.

4) Formation of Referral Network

- 4.1 Upon the constitution of the Local Project Implementers, the STU/S shall extend assistance in the development of the Referral Network for the provision of Auxiliary Services for the beneficiaries of the Project;
- 4.2 The Referral Network shall be formed by identifying potential service providers, whether public or private, that are within the LGU and region, with the use of the prescribed Mapping Tool (Annex B). Such services shall include the following: a) Social Services, b) Health Services, c) Educational Services, d) Social Insurance Services (PhilHealth Coverage); e) Skills Development and Enhancement, and f) Livelihood/ Employment Facilitation Services;
- 4.3 The LSWDO, PDAO with the assistance of the STU/S shall conduct an Orientation on Project among the partners identified during the mapping. The Project Briefer (Appendix D) may also be shared by Local Project Implementers during the Orientation to further the understanding on the Project outcomes, objectives and components;
- 4.4 Institutional arrangements between the Local Project Implementers and the service providers may be crafted with the assistance of the STU/S. The Local Project Implementers, to be collectively represented by the LCE, may forge a formal partnership with the services providers through a Memorandum of Agreement/ Understanding, whichever is appropriate; and

- 4.5 The accomplished Mapping Tool shall be translated into the Directory of Partners and shall be updated by the PDAO every year in close coordination with the LSWDO and/ Case Manager assigned under Project Aruga.

5) Mapping of Inclusive Learning Resource Centers (ILRCs), Therapy and Assessment Centers

- 5.1 The STU/S in close coordination with Local Project Implementers shall identify ILRCs and public or private therapy centers within the area as service providers for professional diagnosis, assessment and appropriate therapies or interventions for children with disability;
- 5.2 The identification shall be conducted with the use of the prescribed Mapping Tool developed by STB;
- 5.3 The STU/S with the assistance of the LSWDO or PDAO shall conduct an Orientation on Project among the partners identified during the mapping. The Project Briefer may also be shared by STU during the Orientation to further the understanding on the Project outcomes, objectives and components;
- 5.4 The STU/S shall highlight among the qualified private and public Assessment and Therapy Centers that the DSWD through its Crisis Intervention Program (CIP) shall initiate the signing of MOA for the release of Guarantee Letters as Medical Assistance for the Project beneficiaries under the Assistance to Individuals in Crisis Situations (AICS);
- 5.5 The STU/S shall forward, through official channels, the list of Assessment and Therapy Centers to STB for proper endorsement of the Cluster Head of the Innovations and Program Development Group to the Cluster Head of Operations Group. The CIP and its regional counterpart, as a member of the PMT, shall perform all the needed undertakings to ensure the successful MOA signing with the identified service provider;
- 5.6 The CIP at the Field Office shall facilitate the notarization of MOA. A Certified True Copy of which shall be furnished to the STU/S and the Local Project Implementers for reference and filing;
- 5.7 All the parties of the MOA shall abide with all its provisions. The monitoring of the compliance of the partner service provider shall all rest with the CIP. The provision of services supported by Guarantee Letters shall be governed by the DSWD Guidelines on AICS. *Refer to DSWD Memorandum Circular (M.C.) No. 20, series of 2025 with the Subject, "Amendment to M.C. No. 16, series of 2022 and M.C. No. 06, series of 2023, of the Assistance to Individuals in Crisis Situations Program" for further guidance;* and

- 5.8 The STU/S in collaboration with the CIP may convene a meeting with the partner ILRC, and public or private Assessment and Therapy Centers for periodic review on the implementation of the AICS provision among Project Aruga beneficiaries. The STU/S may procure the needed logistical requirements, subject to the availability of funding, for the conduct of the activity.

6) Profiling and Assessment of Potential Beneficiaries

- 6.1 The STU/S in close coordination with LSWDO and PDAO shall commence with the house-to-house profiling and assessment of potential beneficiaries using the approved Profiling and Assessment Tool (Appendix E) after the receipt of the signed MOA of all parties concerned;
- 6.2 The list of potential beneficiaries shall be derived from the master list of the Registry of Persons with Disabilities of PDAO and shall be endorsed to LSWDO to determine should the initial list belongs to the poor, vulnerable and disadvantaged sector of the LGU which will be concurred by the *Barangay* Council for the Protection of Children (BCPC);
- 6.3 The LSWDO in coordination with the Local Statistician may also capitalize on their existing data should their CMBS is functional;
- 6.4 Persons with disability who are beyond 18 years of age and who cannot fully take care of themselves due to their diagnosis may still be subjected for profiling and assessment with initial attestation of their impairment by the BCPC;
- 6.5 The STB in coordination with the STU/S shall provide technical assistance to the assigned staff from LSWDO and PDAO on the conduct of profiling and assessment of the potential beneficiaries;
- 6.6 The LSWDO in coordination with STU/S, PDAO and BCPCs shall lead the implementation of the activity and must assign at least one (1) staff who will conduct profiling and assessment of potential beneficiaries in the pilot site of the Project;
- 6.7 All enumerators shall submit a duly approved Non-Disclosure Agreement (NDA) (Appendix F) prior the conduct of the activity. The NDA shall be approved by the designated Data Protection Officer (DPO) or Compliance Officer for Privacy (COP) where the enumerator is affiliated. In the absence of the DPO or COP, the LCE, for LGU-based enumerator, and DSWD Field Office Regional Director, for DSWD Field Office staff, may approve the NDA. The STU/S shall keep the duly approved NDAs for filing and future reference;
- 6.8 The target beneficiaries shall be requested to accomplish the following prior to the interview: a) Consent Form (Annex C), b) Separate Media Release (Annex D), and c) Additional Safeguarding Policy (Annex E). The enumerator shall diligently explain the importance of each of the forms to the respondent.

The Consent Form shall be collected by the enumerator. While the respondent shall be provided with one (1) copy of all the forms accomplished;

- 6.9 The profiling and assessment will not proceed should the respondent refuses to provide Consent;
- 6.10 All of the targeted potential beneficiaries shall undergo an one-on-one interview with the assigned staff from STU/S, LSWDO and PDAO;
- 6.11 A respondent shall only be limited to the parent and/or recognized caregiver of the child diagnosed with disability. In cases that both are absent in the time of the interview, the PDAO in coordination with the BCPC shall schedule the most favorable date of interview; and
- 6.12 All of the profiled potential beneficiaries assessed to be Severe and Moderate shall be subjected for deliberation leading to the prioritization in the final list of potential beneficiaries; and
- 6.13 Those profiled potential beneficiaries who were assessed to be Low and not included in the final list can be assisted under the project through referrals as needed.

7) Deliberations of Potential Beneficiaries

- 7.1 The STU/S in coordination with the Local Project Implementers, shall immediately set a schedule for the deliberations of the profiled and initially assessed potential beneficiaries;
- 7.2 The STU/S shall facilitate the activity and document the deliberation through Minutes of Deliberations. While, the LSWDO and PDAO shall be responsible for the preparation of the presentation of the potential beneficiaries for deliberations;
- 7.3 The deliberation shall be participated in by the representative/s from the STB, STU/S, and Local Project Implementers involved in the profiling and implementation of the Project;
- 7.4 All of the profiled potential beneficiaries with Severe assessment results shall undergo deliberation and will be ranked based their circumstances noted during the house-to-house profiling and assessment;
- 7.5 Should the final list did not reach the total number of target beneficiaries for the current year, the children with disabilities assessed and categorized as Moderate may be considered. While, the children with disabilities assessed and classified as Low may be referred to Auxiliary Services under the Project, as deemed necessary, for possible interventions and shall be maintained in the waitlist for future reference of the DSWD Field Office and Local Project Implementers.

- 7.6 The final list of eligible beneficiaries shall be certified using the prescribed template for the Certification of Eligible Beneficiaries (Annex F) to be signed and endorsed through official channels by the LSWDO and PDAO to the DSWD Field Office;
- 7.7 The endorsement of Persons with Disability, who are beyond 18 years of age, profiled and assessed and is included in the final list of beneficiaries, shall be supported with a Medical Certification from the Local Health Office (LHO);
- 7.8 Upon the receipt of the Certification of Eligible Beneficiaries, the STUs shall immediately notify the LGU through official channels of their inclusion to the Project. Notifying the final beneficiaries shall rest with the PDAO and BCPC to any of their preferred channels of communication; and
- 7.9 In circumstances of demise of any of the endorsed or enrolled beneficiaries, the ranked potential beneficiaries, as waitlisted, from the deliberations shall be considered for formal endorsement by the LGU.

8) Enrollment of Eligible Beneficiaries to Project Aruga Support Management System

- 8.1 Upon the receipt of the acknowledgement of the Field Office on the final list of the Project Aruga beneficiaries, the STU/S shall commence within three (3) working days their enrollment to the Project Aruga Support Management System;
- 8.2 The Project Aruga Support Management System shall only be limited to the following Field Office personnels: a) Innovations Division Chief, b) STU/S Head, and c) STU/S technical staff.
- 8.3 The STU/S shall be responsible for the submission of an NDA (Appendix G) and corporate DSWD Google Suite Account of its users to the STB as a requirement for the issuance of their user credentials. The NDA shall be duly approved by the DSWD Regional Director or Field Offices DPO/ COP;
- 8.4 In circumstances of staff movement or separation from the Field Office, the STU/S shall be responsible in notifying the STB on the matter. New user credentials shall then be issued for the newly onboarded personnel upon the submission of the needed documentary requirements;
- 8.5 The access of users in the Field Office shall only be limited to the data entries and modification under Module 1 of the Project Aruga Support Management System. *Refer to the User's Manual of the Project Aruga Support Management System for further guidance;*

- 8.6 Successful enrollment of the clients to the Information System shall warrant the issuance of a Unique Identification Number and Quick Response (QR) Code;
- 8.7 For beneficiaries from the waitlist and was newly certified to be eligible to the Project shall be immediately enrolled to the Information System (IS) either by the STU/S Head or technical staff upon the receipt of the formal endorsement from the LGU; and
- 8.8 The STU/S shall maintain and secure the beneficiary records in the Information System in accordance with existing confidentiality, records management, and data privacy policies and procedures. *Refer to the implementing rules and regulations of Republic Act No. 10173 or the "Data Privacy Act of 2012" for further guidance.*

9) Mobilization of Auxiliary Caregivers

- 9.1 The LSWDO, PDAO, and LHO with the assistance of the STU/S shall commence with the mobilization of Auxiliary Caregivers in *barangays* within three (3) days after the receipt of formal notice from the DSWD Field Office on the inclusion of the endorsed clients to the Project;
- 9.2 The mobilization of prospective Auxiliary Caregivers may be conducted through the BCPCs, *Barangay* Health Workers (BHW) and Child Development Workers as conduits;
- 9.3 The prospective Auxiliary Caregivers must express volunteerism to become part of the Project implementation and possess the basic knowledge and skills in caring for children. Their expression should be supported by the *barangay* captain through the issuance of *Barangay* Clearance highlighting that the prospective Auxiliary Caregiver is deemed to be of good moral character. Such document shall be submitted to the PDAO for filing;
- 9.4 The Local Project Implementers may conduct screening of the volunteers if deemed necessary;
- 9.5 The LSWDO, PDAO with the assistance of the STU/S shall conduct an Orientation on Project among the prospective Auxiliary Caregivers. The Project Briefer may also be shared by Local Project Implementers during the Orientation to further the understanding of the Project outcomes, objectives and components. Its must also highlighted during the Orientation the ratio of Home Care Support Services that is one (1) Auxiliary Caregiver to ten (10) children with disability;

- 9.6 A Contract of Commitment (COC) (Annex G) shall be forged by the prospective Auxiliary Caregivers after the Project Orientation. Such document shall also be submitted to the PDAO for filling;
- 9.7 The prospective Auxiliary Caregivers who have submitted: a) Barangay Clearance, and b) COC, and has successfully completed the Project Orientation shall be endorsed by the LSWDO to the DSWD Field Office through formal channels using the prescribed template (Annex H) by the STB; and
- 9.8 In circumstances of demise of any of the endorsed Auxiliary Caregivers, the LSWDO, PDAO, and LHO may recruit new volunteers and shall undergo the same process as specified.

10) Capacity Building of Auxiliary Caregivers

- 10.1 The STU/S shall coordinate with the Local Project Implementers on the dates of the Rollout Training of the endorsed Auxiliary Caregivers. The STU/S, LSWDO and PDAO shall be acting as the Learning Management Team (LMT) for the activity;
- 10.2 The STU/S shall lead in the Rollout Training while the members of Local Project Implementers and other professionals within the region may be tapped as Resource Persons (RPs) during the conduct of the activity. The RP's services may be compensated subject to the availability of funding and as applicable. *Please refer to DSWD Administrative Order No. 04, series of 2022 with the Subject, "Amendment to Administrative Order No. 10, series of 2019 or the Guidelines on the Grant of Honoraria to Lecturers, Resource Persons, Coordinators, and Facilitators in Seminars, Training Programs, and other Similar Activities" for further guidance;*
- 10.3 The RPs may also be furnished in advance a copy of the Empowered and Resilient Caregiving Training Manual (Appendix H) and Training Kit for the Trainers (Appendix I) as reference in the conduct of the sessions;
- 10.4 The conduct of the activity should be in accordance to the approved Instructional Design on the conduct of Training on Empowered and Resilient Caregiving (Appendix J) to be made available by the STB among the Field Offices for reference;
- 10.5 The STU/S shall be responsible in recording the entire activity through narrative and photo documentations which will be submitted both to the Office of the Regional Director and STB within 20 calendar days after the activity;
- 10.6 The Auxiliary Caregivers shall be issued with a Certificate of Completion to be signed by the DSWD Field Office Director at the end of the capacity building;

- 10.7 The LMT shall provide the Auxiliary Caregivers who completed the training with Memorandum of Undertaking (Annex I) (MOU) to be concurred by the Heads of LSWDO and PDAO. The MOU shall reaffirm their commitment to the Project and shall be kept for filing by the PDAO;
- 10.8 The LSWDO and PDAO may organize other relevant capacity building activities that aim to further the knowledge and skill sets of the Auxiliary Caregivers. Such activity shall be implemented in close collaboration with the members of the Local Project Implementers that may also serve as RPs;
- 10.9 The STU/S and PDAO shall maintain the list of trained Auxiliary Caregivers for the reference of the DSWD Field Office and LGU, respectively; and
- 10.10 The STU/S shall procure the needed logistical requirements, subject to the availability of funding, for the conduct of the activity.

11) Capacity Building on the Use of Gabay-Magulang Session Guide for Implementers

- 11.1 The STU/S shall coordinate with the Local Project Implementers on the dates of the capacity building on the use of the Gabay-Magulang Session Guide for Implementers. The STU/S, LSWDO and PDAO shall be acting as the LMT for the activity;
- 11.2 The target participants of the training would not be limited to the members of the Local Project Implementers hence it may also be open to Social Workers of the LSWDO and CSO partners engaged in the implementation of the Project in the LGU level;
- 11.3 The STU/S shall lead in the capacity building activity while technical staff from the STB and other professionals within the region may be tapped as Resource Persons (RPs) during the conduct of the activity. The RP's services may be compensated subject to the availability of funding and as applicable;
- 11.4 The RPs may also be furnished in advance a copy of the Gabay-Magulang Session Guide for Implementers (Appendix K) and Trainer's Lecture Kit on Gabay-Magulang Sessions (Appendix L) as reference in the conduct of the sessions;
- 11.5 The STU/S shall be responsible in recording the entire activity through narrative and photo documentations which will be submitted both to the Office of the Regional Director and STB within 20 calendar days after the activity;
- 11.6 A Certificate of Completion to be signed by the DSWD Field Office Director at the end of the capacity building shall be provided among the training completers;

- 10.7 The LMT shall provide the training completers with MOU (Annex J) to be concurred by the Heads of LSWDO and PDAO. The MOU shall reaffirm their commitment to the Project and shall be kept for filing by the PDAO;
- 11.8 The STU/S, LSWDO and PDAO shall maintain the list of trained facilitators of Gabay-Magulang Sessions for the reference of the DSWD Field Office and LGU, respectively; and
- 11.9 The STU/S shall procure the needed logistical requirements, subject to the availability of funding, for the conduct of the activity.

12) Project Orientation with Beneficiaries

- 12.1 The STU/S, in coordination with Local Project Implementers, shall organize and schedule the Orientation of Project beneficiaries in a conducive and accessible venue. The participation of both parents of the children with beneficiaries is ideal but not required;
- 12.2 The Local Project Implementers shall assist in mobilizing the Project beneficiaries and ensure the dissemination of notices and other necessary information related to the activity;
- 12.3 The STU/S shall conduct the Orientation on the Project using the Slides (Appendix M) standardized by the STB. The Project Briefer may also be shared by Local Project Implementers during the Orientation to further the understanding of the Project outcomes, objectives and components.
- 12.4 The STU/S shall highlight during the Orientation that the Disability Support Allowance (DSA) aims to augment the cost of care of the children beneficiaries enrolled under the Project. A Commitment Contract highlighting the same shall be forged by the parent and/ or recognized caregiver and shall be collected by the STU/S for filing;
- 12.5 The STU/S shall be responsible in recording the entire activity through narrative and photo documentations which will be submitted both to the Office of the Regional Director and STB within 20 calendar days after the activity; and
- 12.6 The STU/S shall procure the needed logistical requirements, subject to the availability of funding, for the conduct of the activity.

13. Hiring of Staff Complement for the DSWD Field Offices

- 13.1 The STB shall allocate funding support for the Hiring of nine (9) Social Welfare Officer (SWO) II (SG 15) under Contract of Service (COS) and will be stationed in the STU/S of the DSWD

Field Offices I, II, III, IVA, MIMAROPA, V, VI, XI and National Capital Region;

- 13.2 The STB shall be responsible in crafting the following technical documents in the creation of the said positions: a) Competency-Job Description (Appendix N), b) Request for the Creation of Positions to be approved by the Cluster Head of the Innovations and Program Development Group, and c) Current Staffing of the STB and concerned STU/S;
- 13.3 The STB shall officially transmit all the aforementioned approved documents to the concerned DSWD Field Offices to facilitate the hiring process; and
- 13.4 The hired SWO II shall carry out all the specified terms of the position and the COS may be renewed subject to the availability of funding of the STB for each Fiscal Year.

B. IMPLEMENTATION PHASE

1) Social Case Management of Project Aruga Beneficiary

- 1.1 The children with disabilities assessed to be eligible for the Project shall undergo social case management by the LSWDO through the designated case manager by the LGU. The case load ratio is one (1) social worker to 15 Project Aruga beneficiaries;
- 1.2 The process of Assessment, Treatment Planning, Intervention Monitoring and Termination of the case shall be undertaken by the case manager with the client and his/ her parent and/ or recognized caregiver;
- 1.3 During the process of intake the case manager shall provide a detailed explanation of the client's entry into the social case management within the Project. Forms such as Client's Informed Consent (Annex K) and Data Privacy Consent (Annex L) to be signed any of the child's parents and/ or recognized caregiver shall also be accomplished during the process of intake;
- 1.4 Whole body picture shall be taken by the case manager during the process of intake;
- 1.5 The minimum referrals shall include the provision of Auxiliary Services such as: a) Social Welfare Services, b) Health Services, c) Educational Services, d) Social Insurance Services and e) Skills Development and/ or Enhancement, based on the child's assessed needs;
- 1.6 The Social Case Study Report shall be updated by the case manager every semester and shall be kept together with the: a) child's photo; b) Informed Consent; c) Data Privacy Consent; d) Medical/ Assessment Records, e) Treatment/ Rehabilitation

Plans, and d) Progress Notes accomplished by the case worker and Auxiliary Caregivers, in an individual case folder pursuant to the case management standards set forth by the Standards Bureau;

- 1.7 The STU/S shall diligently monitor the progress of each Project beneficiary on a quarterly basis. All progress shall be duly reflected in the beneficiary's profile in the Project Aruga Support Management System;
- 1.8 The case manager shall administer the Disability Needs Assessment System (DNAS) six months after the intake process. The DNAS shall be readministered once a year to determine the progress of the Project beneficiaries in the gamut of social case management. Results of the annual administration of the DNAS shall be the basis of the case manager to recommend to the members of the Local Project Implementers and DSWD Field Office the exit of the beneficiary from the Project. *Refer to the User's Manual on the administration of DNAS for further guidance;*
- 1.9 The exit from the Project shall be planned with the beneficiary, case manager and DSWD Field Office personnel three (3) months prior to the case termination; hence, this will warrant the delistment of the client to the active payroll generated from the Project Aruga Support Management System;
- 1.10 Reentry to the Project akin to the re-entry of social case management shall be possible in cases that the client garners an unstable level of functioning as a result of the DNAS administration after the beneficiaries' exit. However, the client shall be waitlisted and the timeline of reentry shall depend on the prioritization list of Profiled and Assessed clients and the acknowledgement of the Field Office on the Endorsement of Eligible Project Aruga beneficiaries;
- 1.11 In cases that the Project beneficiary is an active beneficiary of the Pantawid Pamilyang Pilipino Program, Sustainable Livelihood Program (SLP) or Walang Gutom Program (WGP), the case management of the child diagnosed with disability shall rest with the LSWDO while the family members shall be under the supervision of the Municipal/ City Link, SLP or WGP Coordinators, as deemed applicable;
- 1.12 Case conferences shall be organized by the LSWDO and shall be properly documented by the case manager and shall be maintained in the beneficiary's case folder. Case supervisions shall likewise be conducted to safeguard clients, ensure ethical practice, and maintain professional standards on social work practice; and
- 1.13 Sessions for Caring for Carers may be organized by the LSWDO and other members of the Local Project Implementers on a regular basis. The STU/S shall procure the needed logistical

requirements, subject to the availability of funding, for the conduct of the activity.

2) Development Assessment, Rehabilitation and Empowerment of Children with Disabilities (DARE CAREvan)

- 2.1 The STB, in partnership with the National Council on Disability Affairs (NCDA) and the Secretariat of the Regional Council on Disability Affairs (RCDA), shall lead in the implementation of DARE CAREvan in the pilot areas to ensure Early Detection, Prevention and Intervention to Disability (EDPID) among Project beneficiaries. *Refer to the NCDA issuances on the conduct of DARE CAREvan for further reference.*
- 2.2 The STB and NCDA shall meet at least 30 days prior the conduct of each DARE CAREvan to ensure that all the needed preparations are undertaken;
- 2.3 The NCDA and RCDA shall be responsible in tapping professionals such as: 1) medical doctors with diverse specialization, and 2) therapies, that will provide services that are free of charge during the agreed timeline of implementation of the DARE CAREvan;
- 2.4 The STU/S shall coordinate with the pilot areas on the schedules and its counterpart limited to the logistical requirements and assisting staff during the actual conduct of the activity;
- 2.5 The PDAO, BCPCs and BHWs of the LGU shall be responsible in disseminating the conduct of the DARE CAREvan to Project beneficiaries to any communication channels;
- 2.6 All relevant documents as to the diagnosis, evaluation, assessment/s and intervention plans issued by the DARE CAREvan shall be furnished to the designated case manager of the Project beneficiary for the continuum of care; and
- 2.7 The STU/S shall be responsible in recording the entire activity through narrative and photo documentations which will be submitted both to the Office of the Regional Director, NCDA, RCDA members and STB within 20 calendar days after the activity.

3) Provision of Disability Support Allowance

- 3.1 The STU/S in coordination with the Local Project Implementers, through the LSWDO and PDAO, shall organize the monthly or quarterly distribution of Disability Support Allowance (DSA) among the Project beneficiaries on the agreed date and venue for the activity;

- 3.2 The PDAO with the assistance of the Auxiliary Caregivers shall be responsible in disseminating the schedule of the DSA distribution among all the Project beneficiaries;
- 3.3 A flat rate of Two Thousand Pesos (PHP 2,000.00) for each month shall be provided to each child with disability to augment on their cost of care;
- 3.4 The STU/S shall generate the payroll from the Project Aruga Support Management System with the assistance of STB designated IS administrator 15 days prior to the distribution of the DSA. Only the payroll generated from the IS shall be used by the STU/S in the scheduled DSA distribution;
- 3.5 Only the registered claimant, either any of the parents and/or recognized caregiver of the child with disability, in the IS shall be authorized to receive the DSA. In cases of demise of the claimant, the STU/S may amend the entries on the child's profile at the IS subject to the approval of the Innovations Division Chief of the Field Office. A special payroll may be generated with the assistance of the STB designated IS administrator;
- 3.6 A photocopy of one (1) government issued and valid Identification (ID) card with three (3) wet signatures of the claimant shall be the attachment to the payroll. In cases that the claimant has inability to write, a thumbmark shall be considered. The wet signature and thumbmark biometrics should be made by the claimant in front of the Special Disbursement Officer (SDO) or Paymaster;
- 3.7 An MOU (Annex M) highlighting that the DSA's intent on augmenting the cost of care of the Project beneficiaries shall be signed by the claimant on the first scheduled payout and shall be kept by the case manager for filing in the individual case folders;
- 3.8 In circumstances that the claimant has no available valid ID card in time of the distribution of the DSA due to force majeure, a Barangay Clearance, issued within the current month, supported with an updated Case Summary Report by the LSWDO may be considered;
- 3.9 The STU/S must also update the beneficiary's profile in the Project Aruga Support Management System prior to the release of the DSA. In cases of intermittent internet connectivity, the updating of the profile may be conducted by the STU/S within seven (7) working days after the payout;
- 3.10 The Project beneficiaries possessing multiple disabilities shall only be entitled to one (1) DSA for each month;
- 3.11 The STU/S shall organize a special payout within seven (7) working days after the recent release of the DSA to accommodate unclaimed DSAs and special payrolls. Unclaimed

DSAs after this period shall be returned to the Field Office and shall be declared as savings of the STU/S;

- 3.12 The SDO shall immediately liquidate the cash advances made for the DSA not later than three (3) working days from the payout/s to ensure effective and efficient fiscal management. *Refer to DSWD Administrative Order No. 13, series of 2021 with the Subject, "Guidelines on the Rules and Regulations in Granting, Utilizing and Liquidation of all Cash Advances and Penalties for Failure to Liquidate within the Prescribed Reglementary Periods" for further guidance; and*
- 3.13 The STU/S shall be responsible in recording the entire activity through narrative and photo documentations which will be submitted both to the Office of the Regional Director and STB within 20 calendar days after the activity.

4) Assessment/ Evaluation of Project Aruga beneficiaries to Public or Private Centers

- 4.1 The STU/S shall furnish the list of accredited centers to the LSWDO and shall form part of the Referral Network. The list must be updated by the STU/S in coordination with the CIP or its regional counterpart in an annual basis;
- 4.2 The assigned case manager shall refer the Project beneficiaries to any of the public or private centers partnered by the CIP or its regional counterpart should he or she did not benefit from any of the DARE CAREvan conducted or at any point along the case management should he/ she needs further assessment or evaluation;
- 4.3 The referral should be coursed through by the case manager to the DSWD Field Office with an accompanying: a) medical clearance with initial diagnosis with recommendation/s by the LHO, and b) quotation from the accredited service provider. The CIP or its regional counterpart shall then issue a Guarantee Letter (GL) for the services needed that will be provided within three (3) working days after its receipt. The STU/S shall monitor its release within the said time duration;
- 4.4 The release of GL/s for the purpose of the reevaluation or reassessment shall be governed by the existing guidelines of the DSWD on AICS. *Refer to DSWD M.C. No. 20, series of 2025 with the Subject, "Amendment to M.C. No. 16, series of 2022 and M.C. No. 06, series of 2023, of the AICS" for further guidance*
- 4.5 The case manager may accompany the Project beneficiary in during the evaluation or assessment if needed;
- 4.6 The parent and/ or recognized caregiver shall furnish the case manager with the results of the evaluation/ assessment to the

case manager as the basis of the intervention plan during the social case management process. All relevant documents issued by the public or private centers shall be kept by the case manager in the individual case folders of the Project beneficiaries; and

- 4.7 The case manager shall initiate case conferences with the Local Project Implementers for a more responsive provision of interventions, if deemed appropriate.

5) Referral to Therapy Centers

- 5.1 The STU/S shall furnish the list of accredited therapy centers offering behavioral, occupational, physical and speech therapies to the LSWDO and shall form part of the Referral Network. The list must be updated by the STU/S in coordination with the CIP or its regional counterpart in an annual basis;
- 5.2 The assigned case manager shall refer the Project beneficiaries to any of the therapy centers partnered by the CIP or its regional counterpart should he or she did not benefit from any of the DARE CAREvan conducted or its required follow through actions and or at any point along the case management process;
- 5.3 The referral should be coursed through by the case manager to the DSWD Field Office with an accompanying: a) Medical Certificate with diagnosis issued during the DARE CAREvan, and b) quotation from the accredited service provider/s. The CIP or its regional counterpart shall then issue a GL for the services needed that will be provided within three (3) working days after its receipt. The STU/S shall monitor its release within the said time duration;
- 5.4 The release of GL/s for the purpose of the continued therapy sessions shall be governed by the existing guidelines of the DSWD on AICS. *Refer to DSWD M.C. No. 20, series of 2025 with the Subject, "Amendment to M.C. No. 16, series of 2022 and M.C. No. 06, series of 2023, of the AICS" for further guidance.*
- 5.5 Referral to multiple service provider for specific therapy needed by the Project beneficiaries may be made by the case manager subject to the approval of the social worker of the CIP or its regional counterpart;
- 5.6 Referrals to LGU managed therapy centers is paramount to maximize local resources and address accessibility issues among Project beneficiaries;
- 5.7 All relevant documents issued by the public or private therapy centers shall be kept by the case manager in the individual case folders of the Project beneficiaries; and

- 5.8 The case manager shall initiate case conferences with the Local Project Implementers for a more responsive provision of interventions, if deemed appropriate.

6) Referral to Inclusive Learning Resource Centers

- 6.1 The case manager shall initiate the referral of the Project beneficiary to the Inclusive Learning Resource Center (ILRC), based on the assessed needs and the availability of the facility within the LGU;
- 6.2 The ILRC may provide any of the following services to Project beneficiaries, based on their assessed needs: a) Core Educational Services, b) Allied Medical and Therapeutic Services, c) Capacity Building and Support, d) Community and Transition Services, and e) Physical Infrastructure. *Refer to the implementing rules and regulations of Republic Act No. 11650 or "Instituting a Policy of Inclusion and Services for Learners with Disabilities in Support of Inclusive Education Act" for further guidance;*
- 6.3 Individual interventions must be ensured to tailor fit to the needs of the child. Such interventions must be discussed by the case manager and ILRC personnel during case conferences once the child is admitted within the facility;
- 6.4 The personnel from the Department of Education - Schools Division Office (DepEd-SDO), as a member of the Local Project Implementers, shall be responsible in monitoring the Project beneficiary enrolled in the facility in close coordination with the ILRC personnel. The DepEd-SDO shall furnish the case manager with individual reports with progress notes of each enrolled Project beneficiary in a quarterly basis;
- 6.5 All relevant documents issued by the ILRC shall be kept by the case manager in the individual case folders of the Project beneficiaries; and
- 6.6 The case manager shall initiate case conferences with the ILRC personnel/s with the DepEd-SDO for a more responsive provision of interventions, if deemed appropriate.

7) Referral for Skills Enhancement and/ or Development under Technical Education Skills Development Authority (TESDA)

- 7.1 The case manager shall initiate the referral of the Project beneficiaries above 15 years of age to TESDA or TESDA accredited facility for skills enhancement or development, based on their assessed needs;

- 7.2 The services may be accessed from TESDA or TESDA accredited facility are vocational training and National Certification (NC) for Persons with Disability;
- 7.3 The case manager shall assist the parents and/ or recognized caregivers in preparation of the needed documentary requirements for the TESDA training application and scholarship, as deemed applicable;
- 7.4 All relevant documents issued by the TESDA shall be kept by the case manager in the individual case folders of the Project beneficiaries; and
- 7.5 The case manager shall initiate case conferences with the TESDA or TESDA accredited facility personnel/s for a more responsive provision of interventions, if deemed appropriate.

8) Home Care Support Services

- 8.1 The PDAO in coordination with the LHO shall be responsible for the deployment of Auxiliary Caregivers among Project beneficiaries within communities. The ideal case load ratio is one (1) Auxiliary Caregiver to ten (10) Project Aruga beneficiaries;
- 8.2 The Auxiliary Caregivers shall be responsible in assisting the parents and/ or recognized caregivers of Project Aruga beneficiaries on: a) Personal Care, and b) Health Management, to ease their caregiving fatigue that they encounter;
- 8.3 The Auxiliary Caregivers shall be provided with kits (Appendix O) by the DSWD Field Office, subject to the availability of funds, to establish their identity as ancillary workforce under the Project;
- 8.4 The Auxiliary Caregivers must conduct 12 full days of Home Care Support Services for every month to cover all their assigned Project beneficiary. This must be announced among Project beneficiaries with the assistance of the BHW, Child Development Worker and BCPC to ensure the presence of any of the parents/ recognized caregiver or adult member of the household during the home visitation;
- 8.5 The Auxiliary Caregiver shall record his/ her observations on the Project Beneficiary during the home visit through the Progress Notes template (Annex N) supported by a Certificate of Appearance (Annex O), prescribed by STB;
- 8.6 The accomplished Progress Notes shall be submitted for approval by the Auxiliary Caregivers to the PDAO at the end of the month. It shall then be forwarded to the designated case manager for filing in the individual case folder of the Project Aruga beneficiary;

- 8.7 The PDAO shall certify the Services Rendered of all the Auxiliary Caregivers for every month using the template (Annex P), prescribed by the STB, as basis in the release of their monthly honoraria. The Certification shall be endorsed to the STU/S within three (3) working days at the end each month;
- 8.8 The honoraria to be provided to Auxiliary Caregivers shall be based on the approved regional wage order of the National Wages and Productivity Commission (NWPC). *Refer to latest NWPC latest Wage Order per Region for further guidance;*
- 8.9 The STU/S in coordination with the PDAO shall organize the payout of Auxiliary Caregivers either on a monthly or quarterly basis. The STU/S shall prepare the payroll (Annex Q), using the template prescribed by the STB, and the Certification of Services Rendered endorsed by the PDAO;
- 8.10 A photocopy of one (1) government issued and valid Identification (ID) card with three (3) wet signatures of the Auxiliary Caregivers shall be the attachment to the payroll. The wet signature should be made by the claimant in front of the SDO or Paymaster;
- 8.11 In circumstances that the Auxiliary Caregiver has no available valid ID card in time of the payout of the honorarium due to force majeure, a Barangay Clearance issued within the current month may be considered;
- 8.12 The STU/S shall organize a special payout within seven (7) working days for unclaimed honorariums. Unclaimed honorariums after this period shall be returned to the Field Office and shall be declared as savings of the STU/S;
- 8.13 The SDO shall immediately liquidate the cash advances made for the honorariums not later than three (3) working days from the payout/s to ensure effective and efficient fiscal management. *Refer to DSWD Administrative Order No. 13, series of 2021 with the Subject, "Guidelines on the Rules and Regulations in Granting, Utilizing and Liquidation of all Cash Advances and Penalties for Failure to Liquidate within the Prescribed Reglementary Periods" for further guidance;* and
- 8.14 The STU/S shall be responsible in recording the entire activity through narrative and photo documentations which will be submitted both to the Office of the Regional Director and STB within 20 calendar days after the activity.

9) Deployment Social Behavior Change and Communication Materials

- 9.1 Social Behavior Change Communication (SBCC) materials including, but not limited to: a) tote bags, b) t-shirts, c) calendars, c) tumbler, d) stickers, and etc., bearing unified messaging on a *“Barrier-Free Community for Persons with Disability”* intended per target audience will be disseminated among households within the LGU;
- 9.2 Advocacy messages made by the STB and was approved by the Digital Marketing Service may also be disseminated through the official social media platforms of the LGU and members of the Local Project Implementers;
- 9.3 Funding for the reproduction of the SBCC materials will be provided by the DSWD with augmentation support from the LGU through its L/BCPC, LSWDO and PDAO;
- 9.4 Culmination of the advocacy campaigns shall be led by the Local Project Implementers in coordination with the BCPCs and other local partners during the celebration of the National Disability Prevention and Rehabilitation Week and International Day of Persons with Disabilities in the Philippines every third week of July and 03 December of every year, respectively; and
- 9.5 The PDAO with the assistance of the BCPCs shall maintain the list of distributed SBCC materials for the inventory of target audience reached.

10) Conduct of *Gabay-Magulang* Sessions

- 10.1 The parents and/ or recognized caregivers of the Project beneficiaries will be mandated by the LSWDO to attend the *Gabay-Magulang* sessions twice a month within the agreed schedule. The BCPC shall take charge on identifying the most conducive venue within the *barangay* for the conduct of every session;
- 10.2 The BCPC shall take charge on identifying the most conducive venue within the *barangay* for the conduct of every session;
- 10.3 The trained members of the Local Project Implementers and staff from the LSWDO and PDAO on *Gabay-Magulang* Session Guide for Implementers shall be the facilitator of the sessions on a rotation basis. *Refer to the Gabay-Magulang Session Guide for Implementers for further guidance;*
- 10.4 Ideal ratio in the conduct of each session will be one (1) facilitator with 15 parents and/ or recognized caregivers. The participation of both parents is encouraged but not required. However, sessions may be conducted with a maximum of 25 participants per session. Exceeding the said number shall warrant the organization of a separate session group/s;

- 10.5 In the event that there is a low turn-out of parents and/ or recognized caregivers in the progression of the sessions, they may be referred to session groups in other *barangays*. The LSWDO in collaboration with the PDAO may opt to conduct a special session at the LSWDO to meet the minimum target participants for the sessions;
- 10.6 Check-in and check-out activities shall be led by the facilitator for each session to build rapport among the participants and establish a safe space for learning;
- 10.7 The facilitator shall monitor the attendance of the parents and/ or recognized caregivers to each session. The BCPC and Auxiliary Caregivers may be tapped by the facilitator to conduct home visitations to reimpose the parent's participation. Catch up sessions may be conducted in groups or one-on-one basis for the parents who missed any of the sessions;
- 10.8 The LGU, through the available funding of the LSWDO, PDAO or L/BCPC, may incentivise the participation of the parents and/ or recognized caregivers to the Gabay-Magulang sessions to address attrition; and
- 10.9 Disclosures of distress during the conduct of the sessions and needing psychological first aid shall be referred to any trained personnel within the LSWDO.

11) Referral to Existing DSWD Programs of Parents and/or Recognized Caregivers

- 11.1 The designated case manager will provide interventions to the parents and/or recognized caregivers of the Project beneficiaries based on their assessed needs through the administration of the DNAS;
- 11.2 The existing programs of the DSWD that may be provided to the parents and/ or recognized caregivers include:
 - 11.2.1 AICS - parents and/or recognized caregivers may be referred to any of the services under the program and shall undergo the regular process of availment of financial and medical assistance.
 - 11.2.2 Sustainable Livelihood Program (SLP) - parents and/or recognized caregivers may be referred to the SLP - Regional Program Management Office for the livelihood assistance and employment facilitation. The SLP Coordinator shall be responsible in monitoring the progress of the parent and/or recognized caregivers after the provision of the SLP grants; and

- 11.2.3 Therafee Project of the Kapit-Bisig Laban sa Kahirapan-Comprehensive and Integrated Delivery of Social Services (KALAHY CIDDS) - parents and/or recognized caregivers may be referred for Cash-for-Work gearing towards psychosocial support and career development and enhancement interventions.
- 11.3 Upon the receipt of the referral/s of the CIP or its regional counterparts, SLP and KALAHY CIDDS RPMOs, the eligibility screening, assessment and procedures in the provision of services shall be governed by DSWD M.C. No. 20, series of 2025 or the "Amendment to M.C. No. 16, series of 2022 and M.C. No. 06, series of 2023 of the Assistance to Individuals in Crisis Situations", DSWD M.C. No. 03, series of 2025 or the "Omnibus Guidelines on the Implementation of SLP", and "Revised Guidance Note for the Implementation of Cash for Work for Persons with Disabilities under the KALAHY CIDSS - Therafee Project", respectively.
- 11.4 The case manager shall diligently monitor the progress of each referral in close coordination with the STU/S and the assigned AICS worker, SLP coordinator or Therafee focal person; and
- 11.5 All the referrals made by the case manager shall be documented and shall be kept for filing in the individual case folder of the Project beneficiaries.

12) Debriefing Sessions of Project Implementers

- 12.1 The LSWDO shall lead in the provision of debriefing sessions among case managers, Auxiliary Caregivers, assisting *barangay* volunteers, and other members of the Local Project Implementers;
- 12.2 The debriefing sessions shall aim to prevent burnout, promote sustainability, fostering sustainability, fostering a life outside caregiving, and building resilience and meaning;
- 12.3 It may be conducted in a monthly or quarterly basis having partners, who is not involved in the Project implementation, as facilitators or resource persons;
- 12.4 The STU/S in partnership with the LSWDO will also conduct a Caring for Carers Session among case managers, Auxiliary Caregivers, assisting *barangay* volunteers, and other members of the Local Project Implementers, once a year, subject to the availability of funding; and
- 12.5 The PDAO shall be responsible in recording the entire activity through narrative and photo documentations in order to take into account the good practices in the Project implementation.

CHAPTER VI

REPORTING AND DOCUMENTATION

The monitoring and reporting of Project implementation shall be undertaken on a regular basis to ensure compliance with program standards, assess progress in achieving project objectives, identify implementation gaps, and inform necessary technical and operational interventions. The reports generated shall serve as the primary basis for tracking accomplishments, documenting issues and challenges encountered, and recommending appropriate actions to improve project delivery and outcomes.

Monitoring Tool	Frequency of Submission	Responsible Office
Project Implementation Report	Quarterly	Local Government Unit and Social Technology Unit/ Section
Status Report	Semestral	Social Technology Bureau
Year-End Full Implementation Report	Annual	Social Technology Bureau

The Local Project Implementers may also document best practices in the Project Implementation following the Guidelines set forth by the DSWD through Administrative Order No. 05, series of 2016 or the “Good Practice Documentation Guidelines”.

CHAPTER VII

INSTITUTIONAL ARRANGEMENTS

A) Social Technology Bureau

1. Lead in the provision of Technical Assistance and Resource Augmentation (TARA) in the implementation of activities under the Project;
2. Provide technical assistance on the conduct of Learning Development Interventions to Field Office PMT and Local Project Implementers;
3. Develop the Manual of Operations that details the specific procedures of this Pilot Implementation Manual;
4. Organize the National Launching of the Project collaboration with Central Office PMT;
5. Assist in the promotion of the Project to local government units and other stakeholders through the conduct of regular promotion;
6. Recommend impact evaluation of the program to Policy Development Bureau after 5 years of national implementation; and
7. Organize regular and special meetings, as deemed appropriate, for the PMT within the DSWD Central Office.

B) Protective Services Bureau

1. Provide technical assistance to Field Office PMT and Local Project Implementers on case management of children with disabilities; and
2. Attend regular and special meetings of the PMT at the Central Office level.

C) Policy Development and Planning Bureau

1. Lead the Impact Evaluation after five (5) years of Project implementation for enhancements and/ or policy development;
2. Provide technical inputs in the preparation of regional work and financial plans relevant to the Project; and
3. Attend regular and special meetings of the PMT at the Central Office level.

D) Crisis Intervention Program

1. Provide fund augmentation support in the provision of DSA among eligible children with disabilities regularly in line with the existing rules and regulations of Assistance to Individuals in Crisis Situations;
2. Facilitate the MOA partnership with the identified public/private therapy centers in all LGU pilot implementation sites;
3. Verify the validity of documents submitted by Parents and/or Recognized Caregivers prior to the release of the AICS; and
4. Attend regular and special meetings of the PMT at the Central Office level.

E) Sustainable Livelihood Program

1. Provide technical assistance and resource augmentation through the provision of seed capital among parents and/or recognized caregivers of the children with disabilities; and
2. Attend regular and special meetings of the PMT at the Central Office level.

F) Information and Communications Technology Management Service

1. Assist in the operationalization of the Project Aruga Support Management System among Field Office PMT and Local Project Implementers;
2. Provide technical assistance in the maintenance of Project Aruga Support Management System; and
3. Attend regular and special meetings of the PMT at the Central Office level.

G) KALAH-CIDSS

1. Lead in the implementation and operational management of the TheraFee Project under the Cash-for-Work modality for Project Aruga beneficiaries;
2. Provide technical assistance to Local Project Implementers, parents and/or recognized caregivers, and community volunteers on community-driven implementation, documentation, and compliance with project guidelines; and
3. Attend regular and special meetings of the Field Office Project Management Team.

H) Digital Media Service

1. Lead the media relations, promotion, and marketing of the Project through the official platforms of the DSWD;
2. Provide technical assistance to the PMT within the Central Office on the periodic updating of the Project Communication Plan;
3. Provide technical assistance to the PMT within the Central Office on the development and evaluation of Social Behavior Change Communication materials of the Project;
4. Assist in the documentation of events and program milestones; and
5. Attend regular and special meetings of the PMT at the Central Office level.

I) Traditional Media Service

1. Provide technical assistance on communications plans and policies of the Project including the facilitating of livestreams of special events and activities;
2. Lead with the PMT within the Central Office on the development of Social Behavior Change Communication materials of the Project; and
3. Attend regular and special meetings of the PMT at the Central Office level.

J) National Council for Disability Affairs

1. Lead in policy development and advocacy to support the inclusive social protection for children with disabilities and their families;
2. Monitor and coordinate with agencies concerned for the comprehensive intervention for children with disabilities and their families;
3. Maintain an updated registry of children with disabilities in coordination with the Department of Health and the corresponding counterpart offices in the regions;
4. Provide necessary capability-building activities that will support the implementation of the Project;
5. Provide support among children with disabilities through the Developmental

- Assessment, Rehabilitation and Empowerment (DARE) caravan; and
6. Issue a resolution supporting the implementation of Project Aruga and enjoining the provision of technical and support services from PDAO and implementation sites.

K) Department of Health

1. Lead in the enhancement of the Philippine Registry of Persons with Disability as the primary data reference of the Project;
2. Provide technical assistance in the development and implementation of the disability identification and registration in the Philippine Registry of Persons with Disability including the use and roll out of the Harmonized Guidelines on Categorization of Disabilities;
3. Provide rehabilitation services to children with disabilities through Department of Health hospitals;
4. Assist in capacity building of Local Project Implementers especially the Auxiliary Caregivers enabling them to provide the necessary care and services to children with disabilities; .
5. Participate in the establishment and operationalization of the referral networks for children with disabilities and relevant support in the project implementation at all levels.

L) Department of Education

1. Ensure the implementation of inclusive educational services for learners with disability through the implementation of Republic Act No. 11650, series of 2022;
2. Ensure that learners with disabilities are guaranteed their right of access to free public early and basic education services;
3. Advocate referral of children with disabilities enrolled in public schools towards further assessment by health professionals and/or access to social services offered by the Local Social Welfare and Development Office;
4. Ensure the implementation of the Child Find Policy for Learners with Disability Towards Inclusive Education; and
5. Participate in the establishment and operationalization of the referral networks for children with disabilities and relevant support in the project implementation at all levels.

M) Department of the Interior and Local Government

1. Advocate in the creation of PDAO in every province, city and municipality as mandated by Republic Act No. 10070;
2. Specific on children, help ensure that LGUs upgrade/sustain the functionality of PDAO to work for the inclusive social protection of children with disabilities; and
3. Encourage LGUs to include and maintain the social services for children with disabilities in their comprehensive development plans.

N) Philippine Health Insurance Corporation

1. Ensure that children with disabilities shall be granted with immediate eligibility and access to preventive, curative, promotive, rehabilitative, and palliative care;

2. Strengthen the implementation of Z benefits for children with disabilities as a financial risk protection among the marginalized that are enrolled in the Project; and
3. Participate in the establishment and operationalization of the Referral Network/s for children with disabilities and relevant support in the Project implementation at all levels.

O) Council for the Welfare of Children

1. Provide TARA on policy development and implementation of the Project Aruga;
2. Monitor and evaluate the implementation of policies supporting the needs of children with disabilities and their families; and
3. Participate in the establishment and operationalization of the referral networks for children with disabilities.

P) Implementing Local Government Unit

1. Implement the Project as outlined in this Pilot Implementation Manual in close coordination with the PMT of the DSWD Field Office;
2. Oversee the implementation of the Project in collaboration with *barangay* officials and implementers;
3. Allocate necessary funds for the Project including but not limited to representation expenses, venues and etc.;
4. Assist the PMT at the regional level in the creation and maintenance of the Referral Pathway;
5. Assist in the timely and orderly distribution of DSA among parents and/ or recognized caregivers of children with disabilities; and
6. Ensures the periodic updating of Registry of Persons with Disabilities and CBMS as prescribed by existing national rules and regulations.

Appendix A

MEMORANDUM OF AGREEMENT

KNOW ALL MEN BY THESE PRESENTS:

This Memorandum of Agreement made and executed this _____ day of _____ by and between:

The DEPARTMENT OF SOCIAL WELFARE AND DEVELOPMENT through the **SOCIAL TECHNOLOGY BUREAU** a national government agency duly organized and existing under the laws of the Republic of the Philippines, with principal address at DSWD Central Office, Constitution Hills, Batasan Complex, Quezon City, duly represented herein by its UNDERSECRETARY FOR INNOVATIONS AND PROGRAM DEVELOPMENT, **EDUARDO M. PUNAY**, pursuant to Special Order No. 3442, series of 2025 and hereinafter referred to as the “**DSWD**”

-and-

The DEPARTMENT OF SOCIAL WELFARE AND DEVELOPMENT-FIELD OFFICE _____, a national government agency with office address at _____, _____ represented by its Regional Director _____, hereinafter referred to as the “**SECOND PARTY**”;

-and-

The **LOCAL GOVERNMENT UNIT OF** _____, through the **LOCAL SOCIAL WELFARE AND DEVELOPMENT OFFICE (LSWDO)**, with official address at _____, represented herein by Mayor _____, pursuant to Sangguniang Bayan Resolution No. _____ series of _____ and hereinafter referred to as “**LSWDO**”;

And further hereinafter referred to collectively as the “**PARTIES**”.

WITNESSETH THAT:

Whereas, the purpose of this MOA is to establish a formal partnership to jointly implement, monitor, and sustain social protection programs, ensuring services efficiently and effectively reach disadvantaged, marginalized and vulnerable children with disabilities. This shall define the roles and responsibilities of the DSWD, Field Offices, Local Government Units and other identified stakeholders. The Project Aruga has the following components namely: Data Management and Referral Network, Disability Support Allowance, Capability Building, Community Mobilization Support and Auxiliary Services.

Whereas, according to the 2022 study by the Department of Social Welfare and Development (DSWD) and United Nations Children’s Fund (UNICEF) on the Cost of Raising Children with Disabilities in the Philippines, the poverty rate for households with Children with Disabilities is 50% higher than other households, primarily driven by health related expenses and an expenditure requirement that is 40% to 80% higher for children with functional disabilities- representing an equivalence scale of 1.4 to 1.8 such that existing support from disability cards and cash transfers covers only a fraction of the actual 10% to 20% average extra cost incurred at the household level;”

Whereas, the DSWD is the primary government agency mandated to develop, administer, and implement comprehensive social welfare and protection programs and poverty-reduction solutions designed to uplift the living conditions and empower the poor, vulnerable and disadvantaged children, youth, women, older persons, persons with disabilities, families or persons in crisis or at-risk, and communities needing assistance;

Whereas, the DSWD through the STB, developed Project Aruga, is a community-based Project that provides disability inclusive social protection to children with disability and their families to address substantial unmet needs and provide comprehensive social services to provide opportunities towards the development of children with disabilities full potential, which has been assessed after its pilot testing to be Responsive, Effective, Efficient and Sustainable on the needs of the target sector;

Whereas, the DSWD-FIELD OFFICE, as the implementing arm of the DSWD, is mandated to provide technical assistance to Local Government Units (LGUs), Non-Government Organizations (NGOs), Government Agencies (GAs), Academic Institutions, Professional Associations and other members of the civil society to effectively implement programs, Projects and services that will alleviate poverty and empower disadvantaged individuals, families and communities towards an improved quality of life;

Whereas, the DSWD-FIELD OFFICE in attaining its mandate, seeks to establish a collaborative partnership for sharing of resources in the implementation of Project Aruga within CY 2026;

Whereas, the DSWD, DSWD-FO and LSWDO recognize the vital role of the LGUs in providing a direct social service and developmental services to the respond to the needs of children with disabilities;

Whereas, the Implementation Site through the LSWDO in close collaboration with the Persons with Disabilities Affairs Office (PDAO), signifies its full support and willingness to become one of the expansion sites for the Project; and

NOW THEREFORE, for and in consideration of the foregoing premises, the Parties have mutually agreed as follows:

Article I

ROLES AND RESPONSIBILITIES OF THE PARTIES

The First Party (DSWD-Central Office) shall:

1. Agree to review and amend this contract as necessary to ensure its continued relevance and effectiveness
2. Acknowledge their mutual commitment to the faithful and timely compliance with all terms, conditions, and obligations set forth herein.
3. Provide budgetary requirements for the implementation, evaluation and social marketing activities of the Project;
4. Lead in the demonstration of the Project mechanisms in close coordination with concerned agencies, DSWD Office/Bureau/Service/Unit and concerned Field Offices;
5. Develop and/or ensure the availability of a standard module that will be used by the carers and service providers in providing care and support to children with disability
6. Develop key messaging, contents, and layouts of the Social Behavior Change Communication (SBCC) materials and activities based on the approved Project communication plan.

7. Monitor the Project implementation in coordination with the Field Offices through the use of developed monitoring and reporting tools as well as baseline and endline survey among the beneficiaries;
8. Initiate conduct of consultation meetings and Program orientations for the expansion with concerned agencies, partners and stakeholders;
9. Provide technical assistance to the field office and other stakeholders during the implementation of the Project activities;
10. Prepare monitoring reports, documentation of the pilot implementation and conduct of Project evaluation as basis for adoption and/or institutionalization; and
11. Develop and enrich Project guidelines and materials based on the results of the Project expansion.
12. The DSWD through the Social Technology Unit of the Field Offices I, III, IVA, IVB, V, VI, VII, XI and National Capital Region, shall ensure that the roles and responsibilities of the DSWD under this MOA is duly complied with,”

The Second Party (DSWD Field Office) shall:

1. Provide updates to the Third Party on the following:
 - a. Status of the identification of LGU as Project implementation site/s.
 - b. Ensure availability of venues and other logistical requirements for the capability building (if necessary).
 - c. Conduct regular and close monitoring during the Project implementation.
2. Assist in the project implementation and provision of technical assistance
3. Provide necessary technical assistance and resource augmentation to the Third Party to ensure that the basic Project standards and objectives are met;
4. Ensure provision of capacity-building activity and training for the Third Party relative to the Project implementation;
5. Ensure support from concerned Divisions/Units for the Project implementation
6. Coordinate with existing regional mechanisms during the Project Project implementation;
7. Monitor the Project and conduct regular coordination meetings in the course of its implementation;
8. Submit Project implementation reports to the DSWD and produce full documentation of activities
9. Assist in the evaluation/assessment of the Project after all activities are completed through the use of the developed monitoring and reporting tools, as well as baseline and endline survey among the beneficiaries
10. Implement activities at the local government units level as indicated in the approved project guidelines of the DSWD
11. Ensure submission of reportorial requirements to the DSWD , compliant to COA Circular No. 94-013 or the Rules and Regulations in the Grant, Utilization and Liquidation of Funds Transferred to Implementing Agencies.

The Third Party (Local Government Unit / Implementation Site) shall:

1. Form a local Project Management Team and designate focal person through the issuance of a Local Executive Order/ Resolution;

2. Identify beneficiaries and specific communities within the locality where the Project shall be implemented;
3. Participate in all capacity building and other activities related to the Project;
4. Implement activities at the local level as indicated in the Project Guidelines;
5. Ensure provision of the needed support services to the beneficiaries;
6. Ensure submission of required documentation of activities and reports as a result of monitoring and technical assistance conducted;
7. Designate members of the Project Management Team for the implementation of the Project;
8. Designate at least one (1) Social Worker as case manager of the children with disabilities; and
9. Ensure participation of the barangay and the community beneficiaries in community resources mobilization which includes, but is not limited to identification of beneficiaries, capacity building, action planning, implementation, management, project monitoring and evaluation.

Article II DATA PRIVACY AND CONFIDENTIALITY

The Parties agree that the provisions of Republic Act (RA) No. 10173 or the Data Privacy Act of 2012 shall be integrated into this MOA and upheld, and the same principles of transparency, legitimate purpose, and proportionality shall govern the implementation of this MOA. No data or information arising from this MOA shall be disclosed to any third and/or private entity except the government for the purpose of regulation.

A Data Sharing Agreement (DSA) may be entered in compliance with the Data Privacy Act of 2012 to support the execution of this Memorandum of Agreement (MOA) and in the event of the need to share Personal Data or information between the Parties herein. No personal information or data shall be shared between Parties herein unless a DSA shall have been duly executed.

Article III DEFAULT, DELAY, BREACHES

In cases of default, delay or breach, the Defaulting Party shall mean, with respect to the aggrieved party, the occurrence of any one or more of the following:

- a. Any representation or warranty made by the Defaulting Party herein is false or misleading in any material respect when made or when deemed made or repeated and has a material adverse effect on the transactions contemplated by this MOA; and
- b. Failure to perform any material obligation set forth in this MOA, if such failure is not remedied within five (5) working days after the Defaulting Party's receipt of written notice thereof.

Any delay or failure in performance hereunder by either party shall be excused if, and to the extent caused, by occurrences beyond such party's control, including but not limited to restraints of the government, acts of God, force majeure, sabotage or any cause whether similar or dissimilar to those already specified which cannot be controlled by such party.

The Aggrieved Party shall deliver to the Defaulting Party a Notice ("Notice of Default, Delay or Breach") specifying the reasonable detail of the event of default. If within thirty (30) calendar days after the receipt of the Notice of Default, Delay or Breach by the Defaulting Party, the Event of Default has not been remedied, the Aggrieved Party may terminate this MOA and pursue any remedy available to it under this MOA.

If an Event of Default with respect to a Defaulting Party shall have occurred and be continuing, the Aggrieved party shall have the right to:

- i. Designate a day, no earlier than the day such notice is effective and no later than thirty (30) calendar days after such notice is effective, as an early termination date ("Early Termination Date") to accelerate all amounts owing between the Parties and to liquidate and terminate this MOA; and
- ii. Suspend performance

For simple breach: If any Party commits a simple breach, merely fails to comply on time or in the manner agreed upon, without indicating bad faith or gross negligence of any of the terms of this MOA. The aggrieved party must notify the offending party of such breach. The offending party shall then have ten (10) days from receipt of such notice within which to remedy such breach.

For gross violations: If any Party commits gross violation, indicating bad faith or gross negligence of any of the terms of this MOA, or fails to remedy a simple breach within the given period, the aggrieved party may terminate this contract by serving written notice to the offending Party specifically indicating therein the cause of termination.

Article IV VIOLATIONS AND PENALTIES

This MOA shall be implemented in accordance with the terms and conditions herein stipulated. Failure on the part of any party to comply with any provision of this MOA shall warrant its revocation and shall give rise to the initiating appropriate administrative, civil and criminal cases against the erring party.

Article V DISPUTE RESOLUTION

In case of conflict among the Parties, arising from this MOA, all Parties agree to freely and voluntarily submit themselves to necessary consultation and negotiation for purposes of amicable settlement and find a mutually acceptable solution to their dispute within thirty (30) working days from notice by the other Party of such conflict.

Should the Parties fail to reach an amicable settlement of their dispute, the same shall be submitted to arbitration, in accordance with Republic Act (RA) No. 9285 or the Alternative Dispute Resolution Act of 2004. However, should the dispute between the Parties reach the courts of law, the parties agree that the proper courts of Quezon City shall have exclusive jurisdiction over the same.

Article VI INTELLECTUAL PROPERTY RIGHTS

Any intellectual creation or property resulting from the implementation of this MOA shall be subject to and protected according to the Intellectual Property Code of the Philippines and its implementing rules and regulations.

Article VII SEVERABILITY

If any of the provisions of this MOA is found by a court of competent jurisdiction to be void or unenforceable, in the whole or in part, such provision shall be deemed deleted from this MOA but the remaining provisions thereof shall remain in full effect.

Article VIII EFFECTIVITY AND TERMINATION

This MOA may be terminated by any Party upon the service of a written notice at least thirty (30) calendar days prior to the intended date of termination, based on any of the following grounds:

- i. Material breach or violation of any provision of this MOA, including failure of any Party to comply with or perform its obligations;
- ii. Occurrence of force majeure that renders the performance of obligations by any Party no longer feasible;
- iii. Any misrepresentation or false warranty by any Party in connection with this MOA;
- iv. Other analogous circumstances justifying termination.

The termination of this MOA shall not affect the validity, duration, and completion of any Program, activity, or contract that are already executed pursuant to this MOA or by virtue of separate agreements, unless otherwise mutually agreed upon by the Parties.”

IN WITNESS WHEREOF, the parties have caused this contract to be signed in their names on _____ in _____, Philippines.

**FOR THE DSWD through SOCIAL
TECHNOLOGY BUREAU:**

EDUARDO M. PUNAY
Undersecretary

Innovations and Program Development Group

FOR THE DSWD FIELD OFFICE:

[NAME]
Regional Director

DSWD Field Office - [_____]

FOR THE CITY GOVERNMENT OF [LOCAL GOVERNMENT UNIT]

HON. [NAME]
Local Government Unit of [_____]

SIGNED IN THE PRESENCE OF:

HELEN Y. SUZARA
Director IV
Social Technology Bureau

[NAME]
Local Social Welfare and Development Officer
Local Government Unit of [_____]

REPUBLIC OF THE PHILIPPINES)
CITY/MUNICIPALITY OF _____) SS.

ACKNOWLEDGMENT

REPUBLIC OF THE PHILIPPINES)
_____) S.S.

BEFORE ME, a Notary Public for and in _____, on this ____ day of _____, 2026, personally appeared the following with their respective competent evidence of identification:

xxx

known to me and to me known to be the same persons who executed the foregoing instrument and they acknowledged to me that the same is their free and voluntary act and deed as well as that of the entities represented herein.

This instrument refers to a MOA which consists of ____ (__) pages, including the page whereon this acknowledgment is written, each and every page of which, on the left margin, having been signed by the Parties hereto and their witnesses, and sealed with my notarial seal.

WITNESS MY HAND AND SEAL on the date and at the place first above written.

Doc No.: _____;

Page No.: _____;

Book No.: _____;

Series of _____.

Appendix B

TERMS OF REFERENCE OF THE LOCAL PROJECT IMPLEMENTERS UNDER PROJECT ARUGA

I. Rationale

The Social Technology Bureau, as the program development arm of the Department of Social Welfare and Development (DSWD), in collaboration and partnership with United Nations Children's Fund (UNICEF) and the Department of Foreign Affairs and Trade of the Australian government, has launched a disability-inclusive social protection project aimed at addressing the barriers faced by children with disabilities. This initiative is part of the Department's Social Protection Plan for 2023-2028 and rolling work plan for 2023 between DSWD and UNICEF, building on previous research on the challenges faced by children with disabilities in the Philippines.

Pursuant to the DSWD Innovations and Program Development Group M.C. No. 02, series of 2026 that governs the implementation of the Project, the Local Project Implementers, must be constituted that will be composed of key offices within the pilot Local Government Unit and may be expanded to include Civil Society Organizations whose focus is the provision of services to Children with Disabilities.

This Terms of Reference (TOR) is issued to define the composition, functions, and responsibilities of the Local Project Implementers which shall serve as the lead in the local governance structure for the operationalization of the Project.

II. Objectives

The creation of Project Aruga Local Project Implementers shall be created by the pilot Local Government Unit through a local legislation either Executive Order or Local Resolution issued by the *Sanguniang Bayan Lunsod* or Local Chief Executive.

III. Structure and Functions

The Local Project Implementers is established to facilitate the implementation activities of the Project Aruga in pilot areas/ LGU in Regions I, II, III, IVA, MIMAROPA, V, VI, XI and National Capital Region. The LPI shall be led by the Local Social Welfare and Development Office. The minimum members of the LPI shall include:

- Local Social Welfare and Development Office
- Persons with Disability Affairs Office
- Local Health Office
- Schools Division Office
- Public Employment Service Office
- Early Childhood Care and Development

The Roles and Functions of the members of the LPI are as follows:

Local Project Implementers	Roles and Functions
Local Social Welfare and Development Office (LSWDO)	• Designate Case Manager/s, preferably Social Worker, who shall conduct the social case

Local Project Implementers	Roles and Functions
	management of Project beneficiaries; • Participate in the pre-implementation activities of the Project on Profiling and Deliberation of Potential Beneficiaries; • Certify the eligible children with disabilities assessed with severe and moderate case; • Ensures the periodic updating of the Community-based Monitoring System (CBMS) as prescribed by existing national rules and regulations; • Oversee the implementation of the Project in coordination with <i>Barangay</i> officials and volunteers and other local implementers; • Submit the Report on the Project Implementation quarterly to DSWD Field Office in coordination with Persons with Disability Affairs Office; • Responsible for the case management and facilitation of referrals of the identified beneficiaries for appropriate intervention based on the assessed needs; • Lead the facilitation of case conference for the identified special cases of Children with Disabilities; • Conduct of Network Meetings with partner stakeholders; • Lead in the recruitment of qualified Auxiliary Caregivers and their endorsement to the DSWD Field Office; • Participate in all efforts concerning children with disability in the social welfare system, education and health services, and other relevant interventions; • Assist the timely and orderly distribution of Disability Support Allowance among parents and/or recognized caregivers of the Children with Disabilities and honorarium of trained Auxiliary Caregivers; • Allocate necessary funds for the Project including but not limited to representation expenses, venues and etc; and • Review and approve the forwarded progress notes on the conduct of homecare support services to the children with disabilities and their families
Persons with Disability Affairs Office (PDAO)	• Participate in the pre-implementation activities of the Project on Profiling and Deliberation of Potential Beneficiaries; • Provide technical assistance on the registration of children with disabilities in the pilot area; • Ensures the periodic updating of the Registry of Persons with Disabilities as prescribed by existing national rules and regulations; • Provide the masterlist of registered children

Local Project Implementers	Roles and Functions
	with disabilities in the Local Government Unit for targeting of potential beneficiaries of the Project; • Certify the eligible children with disabilities assessed with severe and moderate case; • Issuance of Identification Card to identified beneficiaries of the Project; • Facilitate referrals of Persons with Disability to appropriate agencies or established referral network, based on assessed needs; • Reinforce the implementation of laws and policies concerning Persons with Disabilities in the pilot LGU; • Submit the Report on the Project Implementation quarterly to DSWD Field Office in coordination with Local Social Welfare and Development Office; • Recruit the potential Auxiliary Caregivers for provision of homecare and support services; • Monitor the trained Auxiliary Caregivers and review the submitted progress notes on the conduct of homecare support services to the children with disabilities and their families; • Provide Certification of Rendered Services of the trained Auxiliary Caregivers every month; • Assist the timely and orderly distribution of honorarium of trained Auxiliary Caregivers; • Lead the celebration of International of Persons with Disabilities and National Disability Rights Week; and • Attend Network Meetings with partner stakeholders.
Local Health Office (LHO)	• Provide technical assistance on the implementation of Early Detection and Disability Prevention initiatives in the pilot LGUs, based on assessed needs; • Provide relevant capacity building activities to Auxiliary Caregivers to further their knowledge and skills in the provision of Home Care Support Services; and • Attend regular or special meetings of the Local Project Implementers.
Schools Division / District Office	• Provide technical assistance in accessing Special Needs Education (SNED) and Alternative Learning System (ALS) services

Local Project Implementers	Roles and Functions
	among identified children with disabilities; • Facilitate the enrollment of Project beneficiaries to the Inclusive Learning Resource Center through reasonable accommodation; • Maintain the requirements for a functional Inclusive Learning Resource Center as provided by Republic Act 11650; and • Attend regular or special meetings of the Local Project Implementers.
Local Public Employment Service Office (PESO)	• Provide access to local employment opportunities, trainings, livelihood assistance and other special programs for the parents and/or recognized caregivers of the children with disabilities in the pilot LGU, based on assessed needs; and • Attend regular or special meetings of the Local Project Implementers.
Early Childhood Care and Development	• Provide orientation on Home-based ECCD Program among parents and recognized caregivers; • Act as a Resource Person on Roll-out training on Homecare and Community Support Services to Auxiliary Caregivers; • Promote inclusive education among children with disabilities within the Child Development Center and their transition to Inclusive Learning Resource Center; • Attend regular or special meetings of the Local Project Implementers.

IV. Secretariat

The Local Chief Executive shall designate the Secretariat function to any of the members of the LPI, and shall be responsible on the following:

- Prepare agenda and issue notices for meetings, and ensure all necessary documents requiring discussion or comment are attached to the agenda one week before the meeting;
- Document the proceedings and prepare minutes of the meetings to be distributed to all members one (1) week after the meeting;
- Translate the committee recommendations and technical inputs into action plan for better implementation of the pilot project; and
- Convene the Local Project Implementer team members once in every two (2) months.

]

V. Frequency of Meetings

The Local Project Implementers Team will hold its regular meeting once in every two (2) months. However, the team may hold special meetings as needed. Representation expenses during regular meetings will be funded by the DSWD Field Office subject to the availability of funds and may be augmented by the Local Social Welfare and Development Office or by the host office within the Local Government Unit.

ANNEX B

Appendix C

TERMS OF REFERENCE OF PROJECT MANAGEMENT TEAM OF PROJECT ARUGA

I. Rationale

The Social Technology Bureau, as the program development arm of the Department of Social Welfare and Development (DSWD), in collaboration and partnership with United Nations Children's Fund (UNICEF) and the Department of Foreign Affairs and Trade of the Australian government, has launched a disability-inclusive social protection project aimed at addressing the barriers faced by children with disabilities. This initiative is part of the Department's Social Protection Plan for 2023-2028 and rolling work plan for 2023 between DSWD and UNICEF building on previous research on the challenges faced by children with disabilities in the Philippines.

Pursuant to the DSWD Innovations and Program Development Group M.C. No. 02, series of 2026 that governs the implementation of the Project, a Project Management Team (PMT), shall be constituted composed of the key offices within the DSWD Field Office, whose focus is the provision of services to Children with Disabilities as beneficiaries of the Project.

In addition, the Regional Committee on Disability Affairs, chaired by the DSWD and supported by its member agencies and partners, shall monitor and coordinate the implementation of the Project along the dimensions of remediation, early detection, and rehabilitation of children with disabilities.

This Terms of Reference (TOR) is issued to define the composition, functions, and responsibilities of the PMT at the Field Office level (hereinafter referred to as the "Field Office PMT", which shall serve as the lead regional governance structure for the operationalization of the Project.

II. Objectives

The Field Office PMT is established to mirror the functions of the Central Office PMT at the regional level and to ensure coordinated, accountable, and rights-based implementation of the Project.

The Field Office PMT specifically aims to:

- Provide technical assistance and serve as the regional coordinating body for the implementation of the Project;
- Ensure the delivery of support services consistent with the mandates of member offices and partner agencies in the region;
- Coordinate with implementing Local Government Units (LGUs) and Local Project Implementers in the conduct of project activities, monitoring, and evaluation as indicated in the approved Design; and
- Promote inter-agency complementation, systems thinking, and participatory governance in the delivery of inclusive social protection for children with disabilities.

III. Structure and Functions

The Field Office PMT shall be chaired by the Regional Director of the DSWD Field Office or the Assistant Regional Director for Operations.

Consistent with the mirroring principle established in the M.C., the Field Office PMT

shall be composed of the following:

- Social Technology Unit (lead)
- Protective Services Division,
- Policy Development and Planning Section
- Crisis Intervention Section/ Program
- Sustainable Livelihood Program - Regional Program Management Office
- Kapit-Bisig Laban sa Kahirapan-Comprehensive and Integrated Delivery of Social Services (KALAHI CIDDs) - Regional Program Management Office
- Information and Communications Technology Management Unit
- Communications / Media Relations Unit
- Sectoral Focal for Persons with Disabilities

Membership may be expanded by resolution of the Field Office PMT to include other relevant regional offices and partners, subject to the concurrence of the Chairperson and consistent with the objectives of the Project.

Field Office PMT Member	Functions and Responsibilities
DSWD Field Office	<i>Social Technology Unit (Lead)</i> • Lead in the provision of Technical Assistance and Resource Augmentation (TARA) in the implementation of Project activities at the regional and pilot Local Government Units level; • Provide technical assistance on the conduct of Learning and Development Interventions for Local Project Implementers in pilot Local Government Units; • Lead in the operationalization of the Implementation Guidelines and ensure faithful application of the M.C. within the Field Office; • Convene regular and special meetings of the Field Office Project Management Team; and • Coordinate with the Social Technology Bureau on policy guidance, project design, and reporting requirements. <i>Protective Services Division</i> • Provide technical assistance to Local Project Implementers on case management of children with disabilities; and • Attend regular and special meetings of the Field Office Project Management Team. <i>Policy Development and Planning Section</i> • Provide technical inputs in the preparation of regional work and financial plans relevant to the Project; • Participate in the field evaluation of the Project after its pilot implementation; and

Field Office PMT Member	Functions and Responsibilities
	• Attend regular and special meetings of the Field Office Project Management Team. <i>Crisis Intervention Section / Program</i> • Provide fund augmentation support for eligible beneficiaries, consistent with the existing rules and regulations of the Assistance to Individuals in Crisis Situations (AICS); • Verify the validity of documents submitted by Parents and/or Recognized Caregivers prior to the release of the AICS; and • Attend regular and special meetings of the Field Office Project Management Team. <i>Sustainable Livelihood Program</i> • Provide technical assistance and resource augmentation through the provision of seed capital to parents and/or recognized caregivers of children with disabilities; and • Attend regular and special meetings of the Field Office Project Management Team. <i>KALAH-CIDSS</i> • Lead in the implementation and operational management of the TheraFee Project under the Cash-for-Work modality for Project Aruga beneficiaries; • Provide technical assistance to Local Project Implementers, parents and/or recognized caregivers, and community volunteers on community-driven implementation, documentation, and compliance with project guidelines; and • Attend regular and special meetings of the Field Office Project Management Team. <i>Information and Communications Technology Management Unit</i> • Assist in the operationalization of the Project Aruga Support Management System among Local Project Implementers; • Provide technical assistance in the maintenance of the Project Aruga Support Management System within the region; and • Attend regular and special meetings of the Field Office Project Management Team. <i>Communications / Media Relations Unit</i>

Field Office PMT Member	Functions and Responsibilities
	• Lead the regional media relations and promotion of the Project through the official platforms of the Field Office. • Document events and Project milestones in the region. • Attend regular and special meetings of the Field Office Project Management Team.

IV. Secretariat

The Social Technology Unit of the DSWD Field Office shall serve as Secretariat of the Field Office PMT and shall perform the following functions:

- Prepare the agenda and issue notices of meetings, ensuring that all documents requiring discussion or comment are circulated to members at least one (1) week before each meeting;
- Document the proceedings and prepare the minutes of meetings, to be distributed to all members within one (1) week after each meeting;
- Translate the recommendations and technical inputs of the Field Office PMT into operational plans for the implementation of the Project;
- Maintain the records, registries, and reports of the Field Office PMT, and submit periodic reports to the Central Office PMT through the Social Technology Bureau; and
- Convene the Field Office PMT in regular and special meetings, in accordance with this TOR.

V. Frequency of Meetings

The Field Office PMT shall hold regular meetings once every two (2) months. Special meetings may be called as the need arises, upon the initiative of the Chairperson or upon written request of at least one-third of the members. Representation expenses for regular meetings shall be funded by the Social Technology Bureau subject to the availability of funds under the General Appropriations Act.

Appendix D

PROJECT ARUGA
MAPPING OF POTENTIAL PUBLIC/ PRIVATE THERAPY CENTER PARTNERS

NAME OF THE ENTITY	SERVICES OFFERED	ADDRESS	CONTACT PERSON	CONTACT DETAILS <i>(Contact Number and E-mail address)</i>	REMARKS

ANNEX D

Appendix E

ARUGA PROJECT PROFILING AND ASSESSMENT TOOL

Date of Interview: _____ Time Started: _____ Time Ended: _____

Name of Interviewer: _____

Household ID No.:

Region:

Pantawid Member?

Province:

Name _____ of _____ Respondent:

Address:

CHILD WITH DISABILITY PROFILE

R1.	Name	
R2.	Address	
R3.	Contact Number	
R4.	Date of Birth	
R5.	Religion	Code:
R6.	IP Membership	Code:
R7.	Sex	Code:
R8.	Highest Educational Attainment	Code:
R9.	Disability/ Special Needs	Code:
R10.	Critical Illness	Code:

<u>Religion</u> 0 None 1 Roman Catholic 2 Islam 3 Iglesia ni Cristo 4 Aglipay 5 Seventh Day Adventist 6 Bible Baptist Church 7 Jehovah's Witness 8 United Methodists Church 9 Tribal Religions 10 Other _____	<u>IP Membership</u> <table style="width: 100%;"> <tr> <td style="width: 33%;"> 0 Non-IP 1 Aeta 2 Ati 3 Badjao 4 Bago 5 Batak 6 Bukidnon 7 B'laan 8 Cimaron 9 Cuyonen 10 Dumagat </td> <td style="width: 33%;"> 11 Ibaloi 12 Ibanag 13 Itom 14 Kankanaey 15 Mandaya 16 Mangyan 17 Manobo 18 Palawano 19 Pullon 20 Subanen </td> <td style="width: 33%;"> 21 Tagbanuas 22 Tau't Bato 23 Teduray 24 T'boli 25 Others, please specify: _____ </td> </tr> </table>	0 Non-IP 1 Aeta 2 Ati 3 Badjao 4 Bago 5 Batak 6 Bukidnon 7 B'laan 8 Cimaron 9 Cuyonen 10 Dumagat	11 Ibaloi 12 Ibanag 13 Itom 14 Kankanaey 15 Mandaya 16 Mangyan 17 Manobo 18 Palawano 19 Pullon 20 Subanen	21 Tagbanuas 22 Tau't Bato 23 Teduray 24 T'boli 25 Others, please specify: _____	<u>Sex</u> 1 Male 2 Female
0 Non-IP 1 Aeta 2 Ati 3 Badjao 4 Bago 5 Batak 6 Bukidnon 7 B'laan 8 Cimaron 9 Cuyonen 10 Dumagat	11 Ibaloi 12 Ibanag 13 Itom 14 Kankanaey 15 Mandaya 16 Mangyan 17 Manobo 18 Palawano 19 Pullon 20 Subanen	21 Tagbanuas 22 Tau't Bato 23 Teduray 24 T'boli 25 Others, please specify: _____			
<u>Highest Educational Attainment</u> 1 Without Formal Education 2 Elementary 3 Elementary Graduate 4 High School 5 High School Graduate 6 Vocational Course 7 Voc. Course Graduate 8 College 9 College Graduate 10 Post College Degree	<u>Disability/Special Needs:</u> 0 None 1 Physical 2 Intellectual 3 Learning 4 Visual 5 Mental 6 Psychosocial 7 Deaf/Hard of Hearing 8 Speech and Language impairment 9 Cancer 10 Rare Disease	<u>Critical Illness:</u> 0 None 1 Cancer 2 Cardio-vascular Disease 3 Paralysis 4 Organ Failure 5 Others, specify:			

FAMILY PROFILE

Sinu-sino ang mga miyembro ng inyong pamilya? Ang miyembro ng pamilya ay ang mga indibidwal na kaugnay sa dugo, sa bisa ng kasal o sa pamamagitan ng pag-aampon, o paninirahan sa isang tahanan. Kabilang din ang mga miyembro na hindi kadalasan naninirahan sa tahanan ngunit ang sweldo o gastusin ay kabilang sa buong kita o gastos ng pamilya, *halimbawa*, anak na nag-aaral sa ibang lugar pero ang allowance ay galing sa magulang, miyembro ng pamilya na OFW o nagtatrabaho sa ibang lugar na nagpapadala ng sustento. Ang mga kasama sa tirahan na hindi kaugnay sa pamilya ay hindi kabilang.

H1. Bilang ng Miyembro (Family Size):

	H2.	H3.				H5.	H9.	H10.	
--	-----	-----	--	--	--	-----	-----	------	--

[illegible]

<u>Relationship to the Family Head:</u> 1 Family Head 2 Spouse 3 Offspring 4 Parent 5 Sibling 6 Nephew/Niece 7 Uncle/Aunt 8 Cousin 9 Grandparent 10 Son/Daughter-in-Law 11 Brother/Sister-in-Law 12 Grandchild 13 Father/Mother-in-Law 14 Others	<u>Civil Status:</u> 1 Single 2 Married 3 Cohabitation (Live-in/Common-Law) 4 Divorced 5 Separated 6 Widowed <u>Occupation:</u> 0 Not Applicable 1 Special Occupations 2 Officials of government & special interest orgs, corp. executive, manager, managing proprietor/supervisor 3 Professionals 4 Technicians & Assoc. Professionals 5 Clerks 6 Service worker/shop & market worker 7 Farmers, forestry worker & fishermen 8 Craft and related trade workers 9 Plant/machine operator & assembler 10 Laborers & unskilled workers 11 Others, specify:	<u>Occupation Class:</u> 0 Not Applicable 1 Not employed but looking for work in the past 3 months 2 Not employed and not looking for work in the past 3 months 3 Self-employed 4 Private Company Employee 5 Government Employee 6 Pensioner 7 Overseas Filipino Worker (OFW) 8 Student <u>Solo Parent:</u> 1 Yes 2 No	<u>Disability/Special Needs:</u> 0 None 1 Physical 2 Intellectual 3 Learning 4 Visual 5 Mental 6 Psychosocial 7 Deaf/Hard of Hearing 8 Speech and Language impairment 9 Cancer 10 Rare Disease <u>Critical Illness:</u> 0 None 1 Cancer 2 Cardio-vascular disease 3 Paralysis 4 Organ Failure 5 Others, specify: _____
---	---	--	--

I. Housing, Sanitation and Water

A. Housing Condition

1. What type of construction materials are the roofs and outer walls made of?
 - a. Makeshift/salvaged/improvised materials
 - b. Mixed but predominantly makeshift/salvaged/improvised materials
 - c. Light materials such as bamboo/sawali/ cogon/ nipa but not sturdy and durable
 - d. Mixed but predominantly light materials
 - e. Strong materials such as concrete/brick/ stone or wood or half galvanized iron and half concrete or galvanized iron/aluminum or glass
 - f. Bamboo/ sawali/cogon /nipa but sturdy and durable
2. What is the tenure status of the house and lot does the family have?
 - a. Own house and lot
 - b. Own house, rent-free lot without consent of owner
 - c. Own house, rent-free lot with consent of owner
 - d. Rent-free house and lot without consent of owner
 - e. Rent-free house and lot with consent of owner
 - f. Rented house and lot for less than three years
 - g. Own house, rented lot for less than three years
 - h. Other tenure status, specify: _____
3. Are there any modifications in the house to accommodate the child's disability?
 - a. Yes.
Specify: _____
 - b. No
4. What is the main source of electricity in the dwelling place?
 - a. Electric company
 - b. Generator
 - c. Solar
 - d. Battery
 - e. Others, specify: _____
 - f. None

B. Water Supply

5. What is your family's main source of water supply?
 - a. Unprotected spring, lake, river, rain, dug well
 - b. Commercial sources, e.g., tanker, truck, peddler (except bottled water)
 - c. Source of safe drinking water but the time to collect water including the time to walk to the water source, collect it and return is longer than 30 minutes
 - d. Own use faucet community water system (*gripa*)
 - e. Shared faucet community water system
 - f. Own use tubed/piped deep well
 - g. Shared tubed/ piped deep well, tubed/piped shallow well
 - h. Bought bottled water including those being delivered
 - i. Others, specify: _____

C. Sanitation

6. What is the main type of toilet facility the family uses?
 - a. Water-sealed, sewer septic tank, used exclusively by the family
 - b. Water-sealed, sewer septic tank, shared with other families
 - c. Closed pit

- d. Open pit
 - e. Others (pail system, and others), specify: _____
 - f. None
7. Is the toilet accessible for the child?
- a. Yes
 - b. No
8. What is the main system of garbage disposal adopted by the family?
- a. Garbage collection
 - b. Burning
 - c. Composting
 - d. Recycling
 - e. Waste Segregation
 - f. Pit with cover
 - g. Pit without cover
 - h. Throwing of garbage in rivers, vacant lots, etc.
 - i. Others, specify: _____

II. Health

A. General Health

1. Has the child received all recommended vaccinations?
- a. Yes
 - b. No
2. Does the child have any ongoing health conditions?
- a. Yes.
Specify: _____
 - b. No

B. Health Expenses

3. Please provide the corresponding health expenses in a month for each of the following areas, as applicable:

Areas of concerns		Total (per month)	Remarks
Food (e.g. milk, cookies, oatmeal, etc.)			
Medication	• Maintenance		
	• Vitamins		
	• Other related medicines		
Therapy (physical, occupational, speech)			
Hygiene-related needs			
Other health needs			
Over-all Total			

C. Access to Health Services

4. Has the child availed health services in the past 6 months?
 - a. Yes.
What health services availed? _____
 - b. No.
5. Is the health facility accessible for the child?
 - a. Yes
 - b. No
6. Are there any barriers to accessing health care services (e.g., transportation, consultation fees, etc.)
 - a. Yes.
Specify: _____
 - b. No

III. Education

A. Educational Status

1. Is the child currently enrolled in school?
 - a. Yes
Grade/Year Level: _____
 - b. No
Why not? _____

B. School Accessibility

2. Is the school equipped with physically accessibility features (ramps, handrails and grab bars, etc.)
 - a. Yes
Specify: _____
 - b. No
 - c. Not Applicable
3. Are there special education programs available?
 - a. Yes
Specify: _____
 - b. No
 - c. Not Applicable
4. Does the child receive any learning support (special education teacher, assistant, etc.)
 - a. Yes
Specify: _____
 - b. No
 - c. Not Applicable

C. Learning Materials

5. Are there accessible learning materials available for the child (braille, audio books, etc.)?

- a. Yes
Specify: _____
 - b. No
 - c. Not Applicable
6. Does the child have access to assistive technology (hearing aids, communication devices, etc.)?
- a. Yes
Specify: _____
 - b. No
 - c. Not Applicable

IV. Economic Capacity

A. Family Income

1. What is the primary source of income for the family?

2. How much is the approximate monthly income of the family? _____
- Below minimum wage
 - Minimum wage
 - Above minimum wage

B. Employment

3. Are the parents/guardians employed or have entrepreneurial activities?
- a. Yes
Specify: _____
 - b. No
If not, what is the reason for unemployment?
 - Lack of skills
 - Health reasons
 - Caregiving responsibilities
 - Others. Specify _____

V. Service Availment

A. Social Services

1. Does the family receive any form of financial assistance (government subsidies, medical assistance)?
- a. Yes
Specify: _____
 - b. No
2. Is the family aware of available social services for children with disabilities?
- a. Yes
Specify: _____
 - b. No
3. Has the family availed of any services (therapy, financial support, special education programs, etc.)
- a. Yes
Specify: _____

b. No

B. Barriers to Service Availment

4. What are the challenges faced in availing these services?

- Lack of information
- Distance and transportation
- Financial constraints
- Prejudice and discrimination
- Others, specify: _____

VI. General Observations and Recommendations

A. Strengths: (e.g., supportive family, strong community network)

B. Assessment:

C. Recommended Actions/Interventions:

Scoring and Interpretation

The assessment can be evaluated based on a scoring system to prioritize areas that require immediate action:

Rating Scale	Description
1	Severe: The well-being of the child and family is at a critical level, requiring urgent and immediate intervention . The situation poses serious risks, such as unmet basic needs, lack of access to essential services (e.g., healthcare, education, therapy) Examples: • The child is unable to access critical medical or therapeutic care. • The family is experiencing severe financial instability, threatening, endangering, jeopardizing basic needs like housing or food.
2	Moderate: The well-being of the child and family shows significant areas of concern that need to be addressed in the short term. While the situation is not as dire as severe cases, it could worsen if left unresolved. Improvement efforts are necessary to prevent escalation. Example: • The child has limited access to required services, or current services are inconsistent or inadequate.
3	Low: The child and family's well-being is generally adequate , with some minor

	issues or vulnerabilities that require attention but do not pose immediate risks. Regular monitoring and proactive measures can help maintain stability and prevent regression. Examples: • The child is receiving most necessary services but may need better access to specific supports (e.g., speech therapy, social inclusion programs). • The family is functioning well overall but faces occasional challenges (e.g., financial strain, minor caregiver stress).
4	Stable: The child and family's well-being is satisfactory and functioning as expected, with no major concerns. They have access to appropriate services, and their needs are met effectively. Regular monitoring ensures continued stability and progress. Examples: • The child is receiving consistent care and education tailored to their needs, with positive outcomes. • The family has a strong support network, sufficient resources, and effective coping strategies.

1 - Severe: Immediate intervention needed

2 - Moderate: Address within a short period

3 - Low: No immediate action needed, but regular monitoring

4 - Stable: Meets all needs effectively

Appendix F

NON DISCLOSURE AGREEMENT

FOR GOOD CONSIDERATION, and in consideration of my service (professional and voluntary) with the Local Government Unit of **[NAME OF LGU/ DSWD FIELD OFFICE]** under the Project Aruga, the undersigned hereby agrees and acknowledges:

- 1) That during the course of my contract, there may be disclosed to me certain protected data of the LGU such as but not limited to:
 - Personal information: any information whether recorded in a material form or not, from which the identity of an individual is apparent or can be reasonably and directly ascertained by the entity holding the information, or when put together with other information would directly and certainly identify an individual.
 - Sensitive personal information refers to personal information:
 - About an individual's race, ethnic origin, marital status, age, color, and religious, philosophical or political affiliations and type/s of disability/ies;
 - About an individual's health, education, genetic or sexual life of a person, or to any proceeding for any offense committed or alleged to have been committed by such person, the disposal of such proceedings, or the sentence of any court in such proceedings;
 - Issued by government agencies peculiar to an individual which includes, but not limited to, social security numbers, previous or current health records, licenses or its denials, suspension or revocation, and tax returns; and
 - Specifically established by an executive order or an act of Congress to be kept classified
- 2) I agree that I shall not during, or at any time after the termination of my contract with the **[NAME OF LGU/ DSWD FIELD OFFICE]**, use for myself or others, or disclose or divulge to others including future employees, any protected data, confidential information with or without label, or any other proprietary data of the **[NAME OF LGU/ DSWD FIELD OFFICE]** in violation of this agreement.
- 3) I agree that I shall follow/ observe the general guidelines when dealing with personal data as stated in the Data Privacy Act as specified in the Project's Enhanced Pilot Implementation Guidelines:
 - The only people able to access data covered by this policy should be those who need it for social case management.
 - Data should not be shared informally. When access to confidential information is required, staff can request it from their designated Data Protection Officers.
 - Project Aruga staff should keep all data secure, by taking sensible precautions and following the guidelines below.
 - a. Personal data that are digitally processed should be encrypted, whether at rest or in transit. Passwords or

passphrases used to access personal data should be of sufficient strength to deter attacks, and they should never be shared.

- b. Transfers of personal data via electronic mail shall use a secure email facility with encryption of the data, including any or all attachments.
- c. Use of facsimile technology is not secure and should not be used for transmitting documents containing personal data.
- d. Personal data should not be disclosed to unauthorized people, either within the organization or externally.
- e. Data should be regularly reviewed and updated if it is found to be out of date.
- f. If no longer required, it should be archived, deleted and/or disposed of.
- g. Project Management Team/ Local Project Implementers should request help from their designated data protection officer if they are unsure about any aspect of data protection.

4) That upon the termination of my contract from the LGU name:

- I shall return to the **[NAME OF LGU/ DSWD FIELD OFFICE]** all documents and property of the **[NAME OF LGU/ DSWD FIELD OFFICE]**, including but not necessarily limited to: accomplished forms, reports, manuals, correspondence, clients' data, computer programs, and all other materials and all copies thereof relating in any way to the LGU/ Field Office operation, or in any way obtained by me during the course of my contract. I further agree that I shall not retain copies, notes or abstracts of the foregoing.
- The **[NAME OF LGU/ DSWD FIELD OFFICE]** may notify any future or prospective employer or third party of the existence of this agreement, and shall be entitled to full injunctive relief for any breach.

5) This agreement shall be binding upon me and my personal representatives and successors in interest, and shall ensure to the benefit of the **[NAME OF LGU/ DSWD FIELD OFFICE]**, its successors and assigns; any breach of this agreement may be regarded as an infringement of the terms of the contract.

- The **[NAME OF LGU/ DSWD FIELD OFFICE]** shall be entitled to seek remedies in the event that I have breached this agreement, or threatens to, in whole or in part. In such a case, the **[NAME OF LGU/ DSWD FIELD OFFICE]** may enter into an amicable settlement with me. The **[NAME OF LGU/ DSWD FIELD OFFICE]**, however, reserves the right to pursue other remedies, including a claim for losses and damages should it determine that an amicable settlement would not serve its best interests.
- That by affixing their signature, the Data Privacy Officer _____ and/or DSWD Regional Director of the _____ shall ensure that the undersigned shall abide by this Non-Disclosure Agreement and by the principles and provisions of the Data Privacy Act of 2012.

IN WITNESS WHEREOF, the Parties have indicated their acceptance of this Agreement by their signatures below on the dates specified.

Conforme:

Signature over Printed Name

Date:

Attested by:

Signature over DPO/ DSWD RD's Name

Date:
